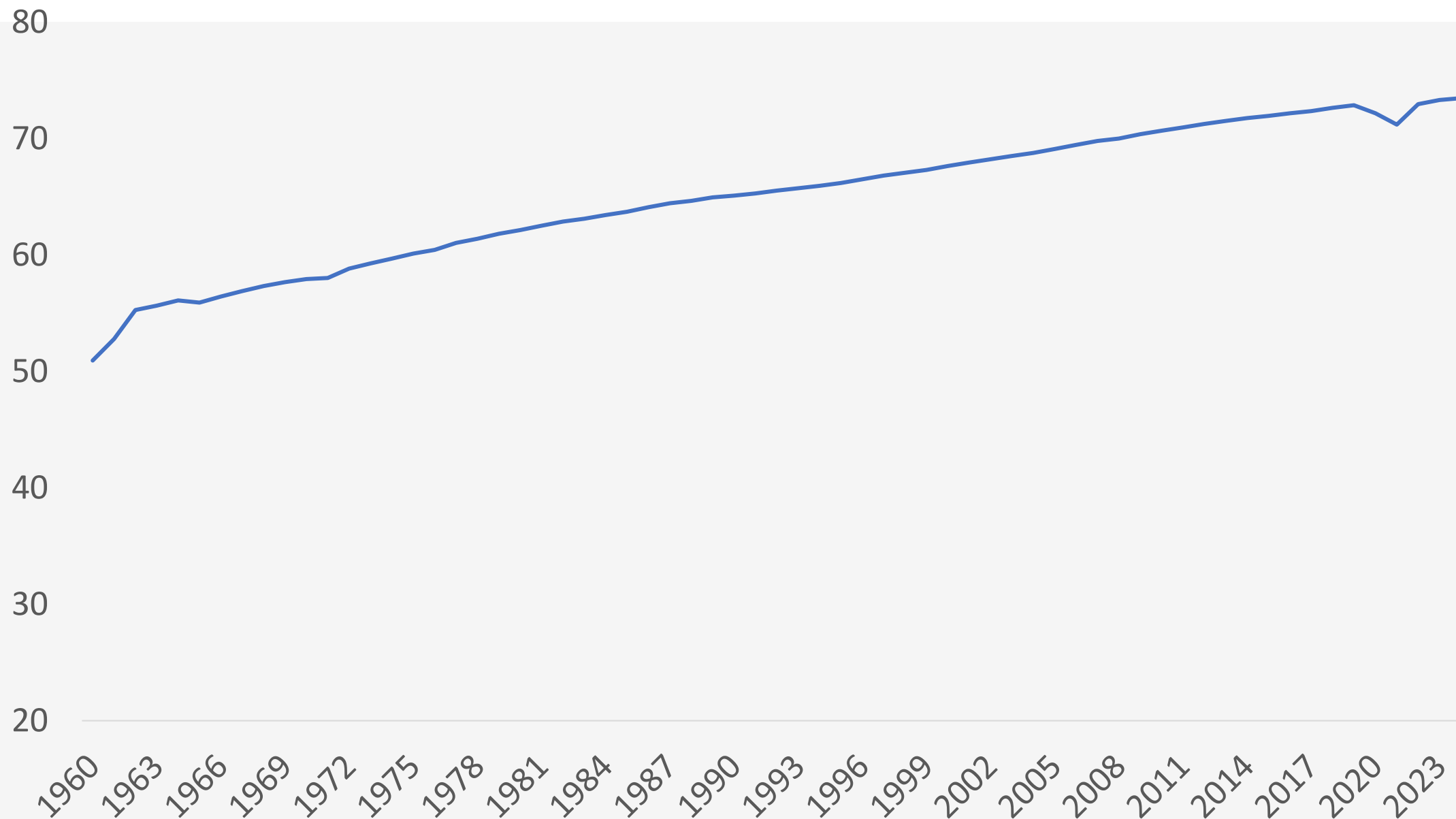


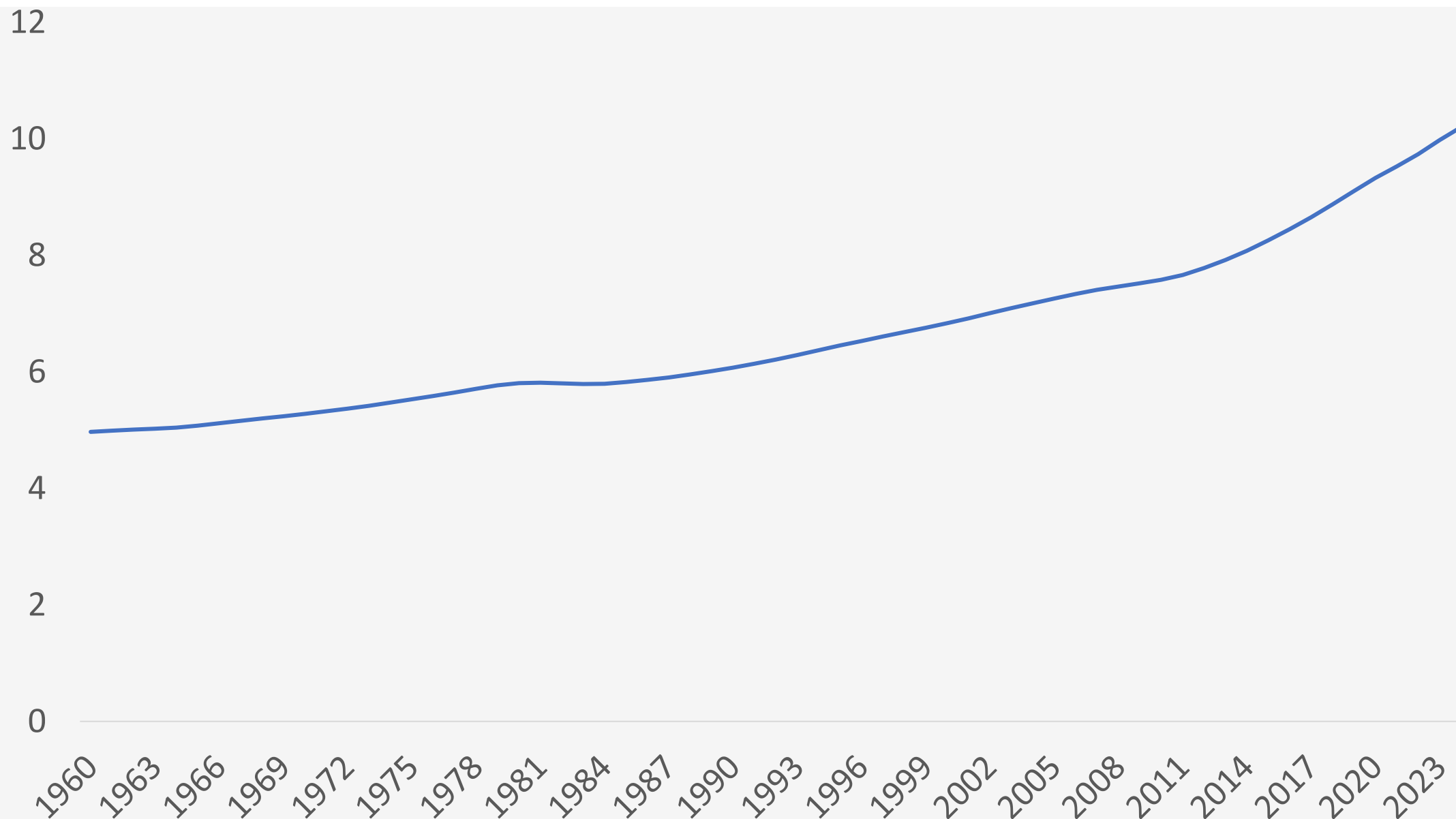
Population aging and intergenerational relationships

Day 3: Health Implications of Population Aging

Life expectancy at birth, total (years)

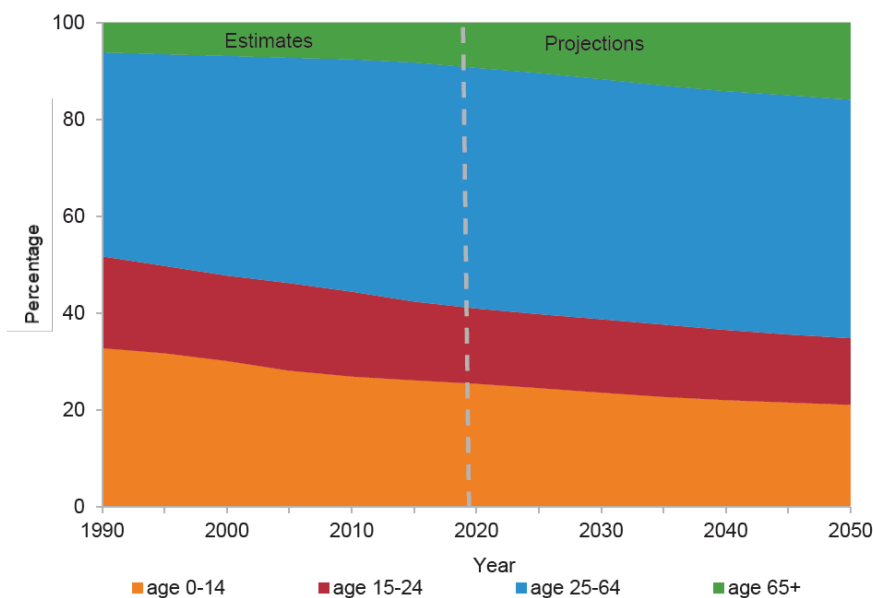


Population ages 65 and above (% of total population)



POPULATION AGING IS AN UNDENIABLE GLOBAL PHENOMENON

Figure 2.
Global population by broad age groups, 1990-2050 (percentage)



Source: United Nations, Department of Economic and Social Affairs, Population Division (2019). *World Population Prospects 2019*.

Table 1.
Number of persons aged 65 years or over by geographic region, 2019 and 2050

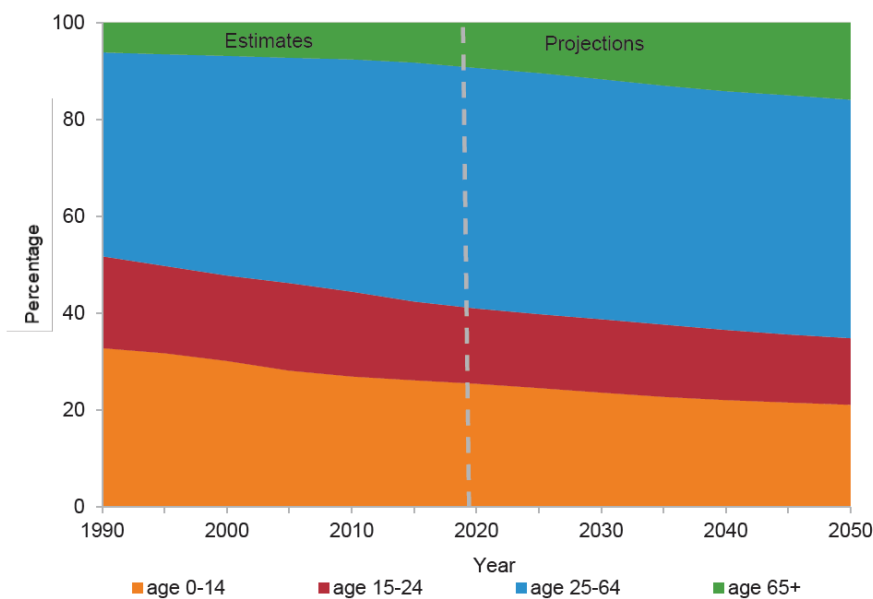
Region	Number of persons aged 65 or over in 2019 (millions)	Number of persons aged 65 or over in 2050 (millions)	Percentage change between 2019 and 2050
World	702.9	1548.9	120
Sub-Saharan Africa	31.9	101.4	218
Northern Africa and Western Asia	29.4	95.8	226
Central and Southern Asia	119.0	328.1	176
Eastern and South-Eastern Asia	260.6	572.5	120
Latin America and the Caribbean	56.4	144.6	156
Australia and New Zealand	4.8	8.8	84
Oceania, excluding Australia and New Zealand	0.5	1.5	190
Europe and Northern America	200.4	296.2	48

Source: United Nations, Department of Economic and Social Affairs, Population Division (2019). *World Population Prospects 2019*.

*Excluding Australia and New Zealand.

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We are living longer,
but are we living
better?

What are the
implications of
aging for healthcare
systems?

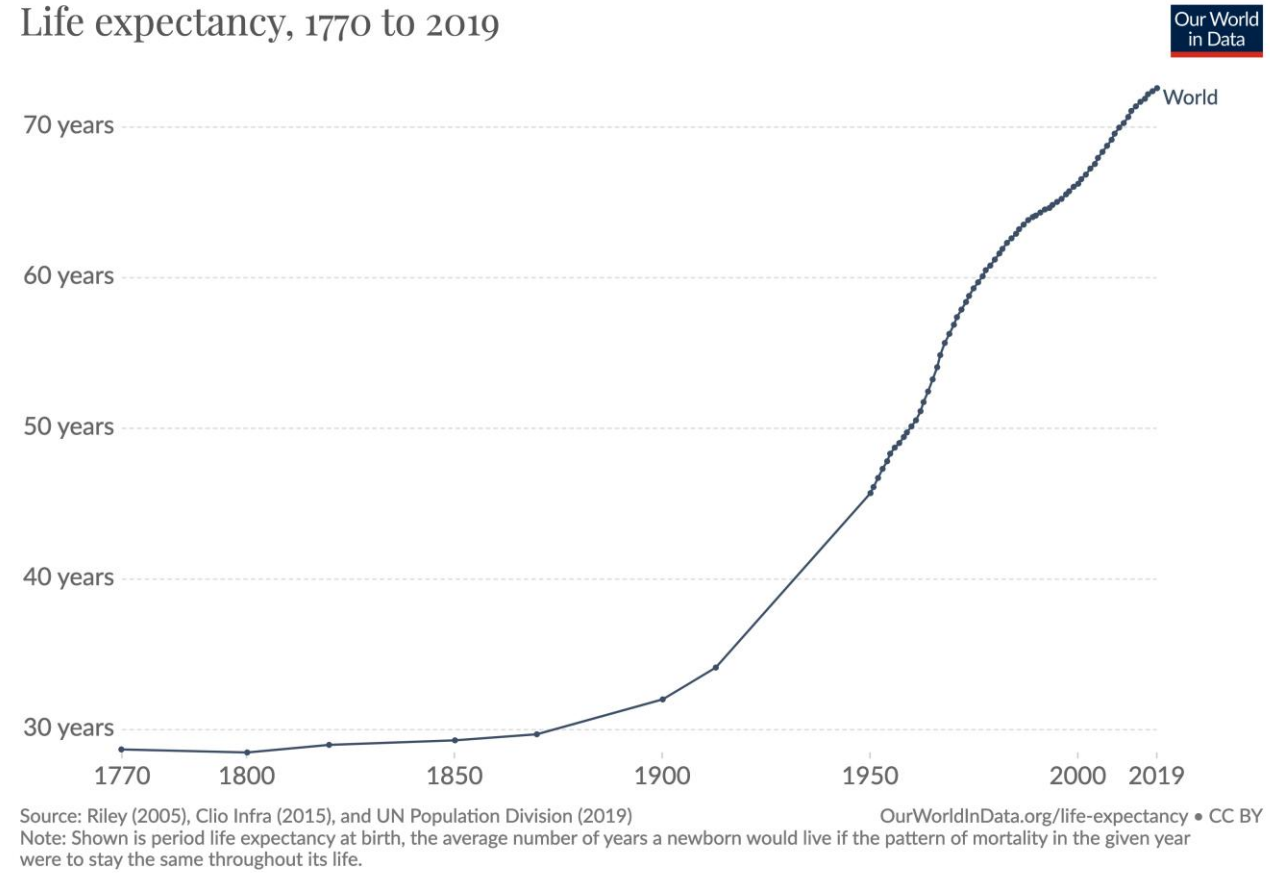
MORTALITY TRANSITION

For most of history, human life expectancy was short: in hunter-gatherer societies it was about 25 years, while in England in 1700 it was only 37 years (Cutler, Deaton, & Lleras-Muney, 2006).

Beginning in the 18th century, mortality began to decline in developed countries.

Throughout the 20th century, there was a historically unprecedented decline in mortality, leading to an increase in life expectancy of approximately 30 years.

Life expectancy, 1770 to 2019



DETERMINANTS OF THE DECLINE IN MORTALITY

Nutritional
improvements

Advances in
public health

Vaccination

Advances in
medical
treatments

Urbanization

Early life care

Living
conditions

EPIDEMIOLOGICAL TRANSITION

The concept of "epidemiological transition" was proposed by Omran (1971) to indicate the progressive evolution in patterns of death, morbidity, and disability.

In general, these changes occur concomitantly with other demographic, social, and economic transformations (Horiuchi, 1997).

EPIDEMIOLOGICAL TRANSITION

- (1) the substitution among the main causes of death from common infectious diseases to non-infectious diseases, such as degenerative and cardiovascular diseases;
- (2) the shift of the greatest burden of morbidity and mortality from younger age groups to older age groups;
- (3) the declining dominance of mortality counterposed by an increasing dominance of morbidity.

EPIDEMIOLOGICAL TRANSITION

Horiuchi (1997) proposes a five-stage model to explain the epidemiological transition of human mortality.



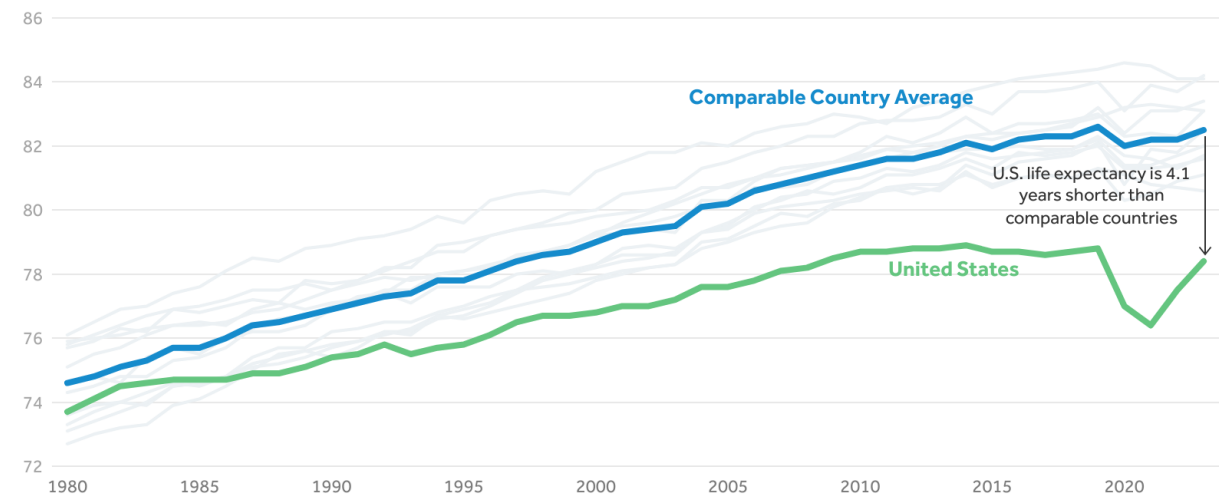
1. Reduction in mortality from external causes.
2. Reduction in mortality from infectious, maternal, and perinatal diseases and nutritional deficiencies.
3. Decrease in deaths from cardiovascular diseases.
4. Decrease in cancer mortality.
5. Slowing of senescence.

REVERSE TRANSITIONS

According to Horiuchi (1997), reverse transitions can lead to increased mortality due to:

1. Poor working conditions, inadequate food, housing, and sanitation;
2. Less healthy lifestyles;
3. Emergence of new infectious diseases (e.g., AIDS).
4. Impacts of pollution, environmental degradation, and exposure to harmful agents.
5. Social alienation (depression, increased homicides and suicides)

Life expectancy at birth, in years, 1980-2023



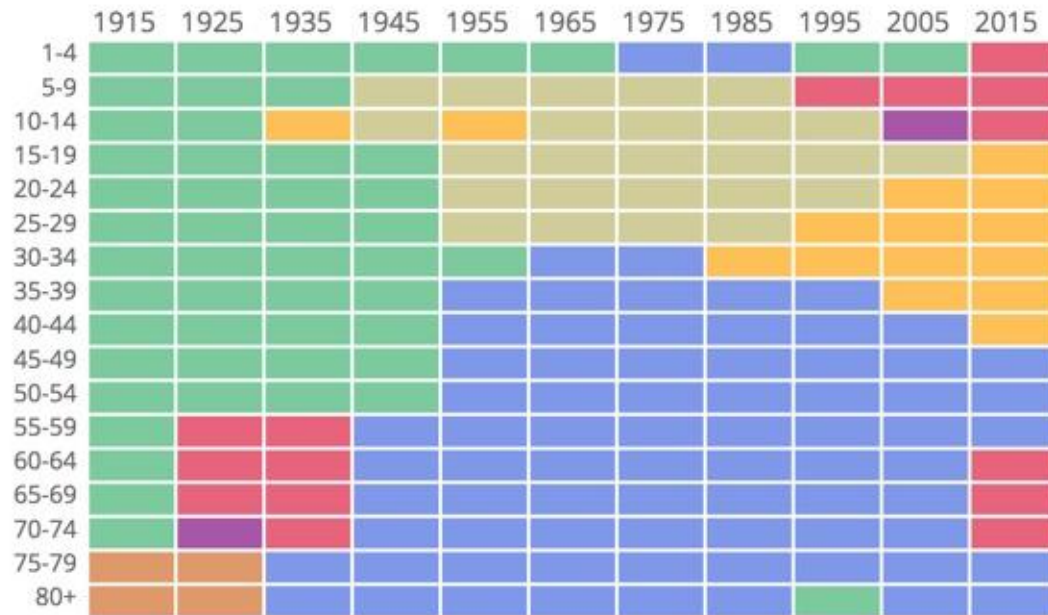
Notes: Comparable countries include Australia, Austria, Belgium, Canada, France, Germany, Japan, the Netherlands, Sweden, Switzerland, and the U.K. 2023 U.K. life expectancy data is only for England and Wales. See Methods section of "How does U.S. life expectancy compare to other countries?"

Source: KFF analysis of CDC, OECD, Australian Bureau of Statistics, German Federal Statistical Office, Japanese Ministry of Health, Labour, and Welfare, Statistics Canada, and U.K. Office for National Statistics data

Peterson-KFF
Health System Tracker

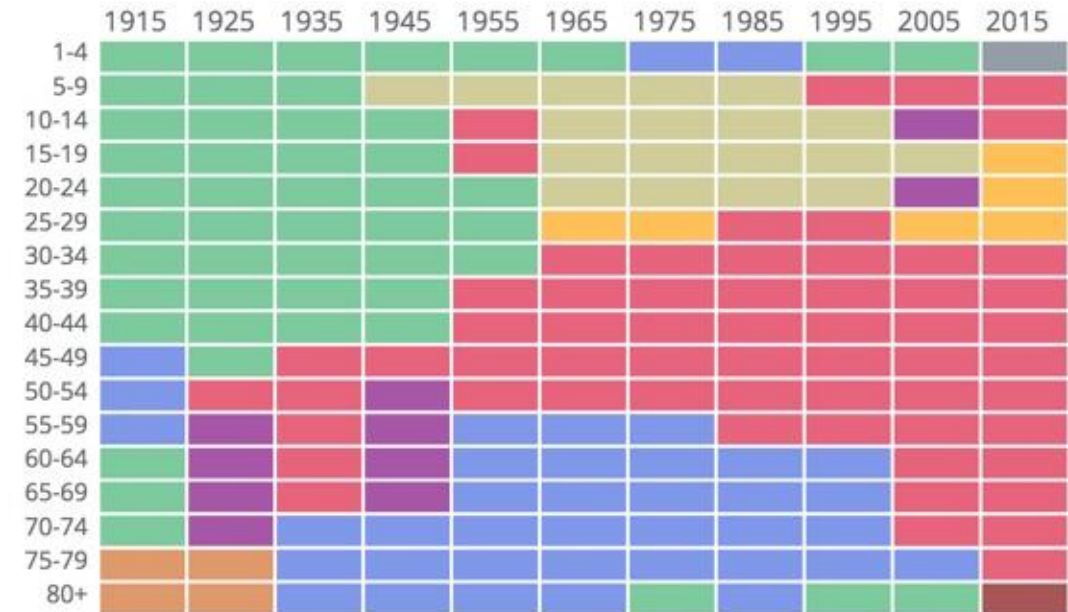
Most common cause of male death:

Total



Most common cause of female death:

Total



Tiles Type

Green	Infections
Pink	Cancer
Blue	Heart conditions
Yellow	External
Olive	Motor vehicle incidents
Purple	Conditions of the nervous system
Brown	Dementia
Orange	Senile decay
Grey	Other

Causes such as:

Tuberculosis, Bronchopneumonia
Malignant neoplasm of breast, lung, brain, etc.
Myocardial infarction, coronary disease
Drug misuse, suicides, self-harm
Collisions with vehicles, cycles, pedestrians
Cerebral palsy, Epilepsy
Unspecified dementia
Senile decay, other forms of senile decay
Other ill-defined and unspecified causes

Source: UK Office of National Statistics

<https://www.cambridgemedchemconsulting.com/news/index/files/0a5e6b7dc051e5bf64a6acf067cea9d2-292.html>

PROSPECTS FOR MORTALITY DECLINE?

- in which age groups?
- in which causes?
- convergence/divergence?



CRITIQUES OF THE CLASSICAL THEORY OF EPIDEMIOLOGICAL TRANSITION

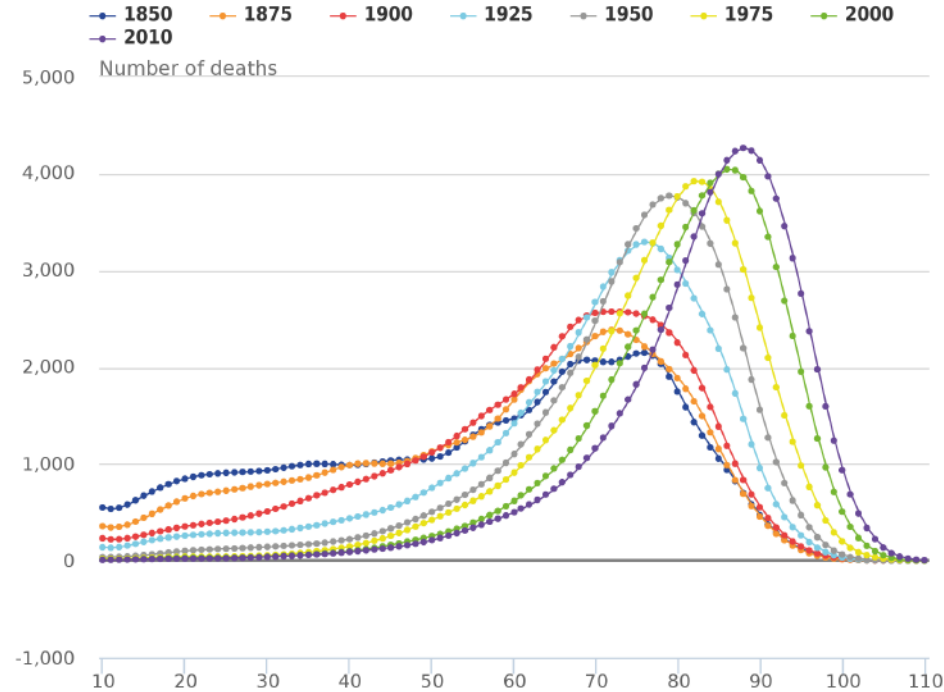
The classical theory of Epidemiological Transition, formulated by Omran (1971), posits a linear and progressive shift in mortality patterns, from infectious to chronic diseases.

- **Olshansky and Ault (1986):** introduced the concept of the "age of delayed degenerative diseases," highlighting that gains in longevity involve postponing the onset of chronic conditions, compressing morbidity into later life stages.
- **Vallin and Meslé (2004):** health trajectories vary among nations, with different *convergence and divergence waves* of mortality decline impacting populations unequally.
- **Frenk et al. (1991):** the epidemiological transition in Latin America follows a "polarized-prolonged model", with an overlap of high incidences of pre-transitional stage diseases and post-transitional diseases—without a clear resolution of each stage of the transition process.

RECTANGULARIZATION-COMPRESSION OF THE MORTALITY CURVE

Figure 6: Number of Deaths by Age from the Period Life Tables, England and Wales, Females

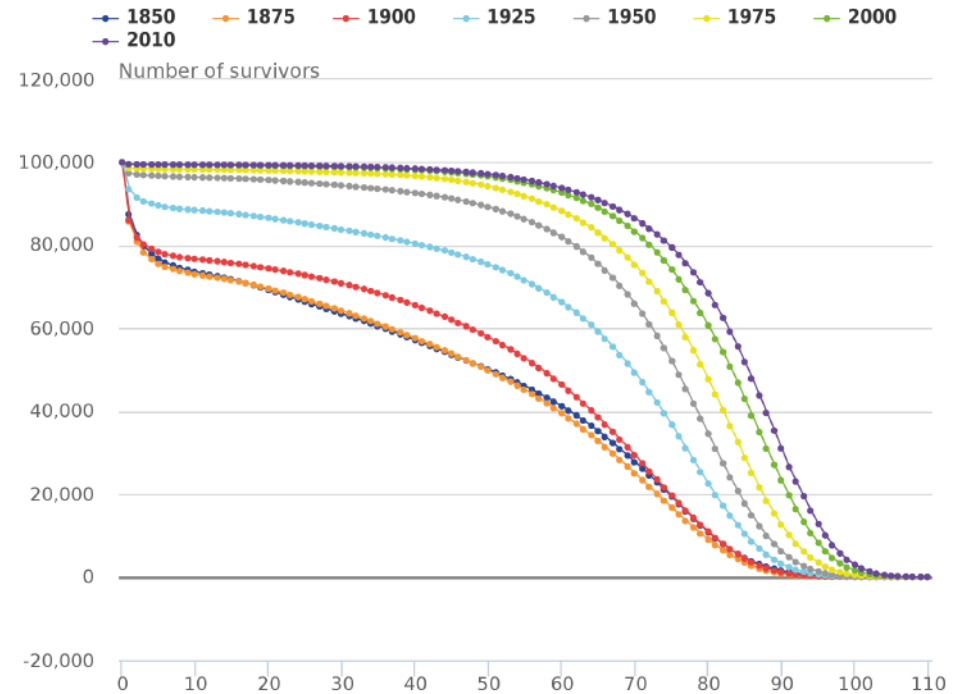
Selected Years



Source: Office for National Statistics

Figure 4: Number of Survivors by Age, from the Period Life Tables, England and Wales, Females

Selected Years



Source: Office for National Statistics

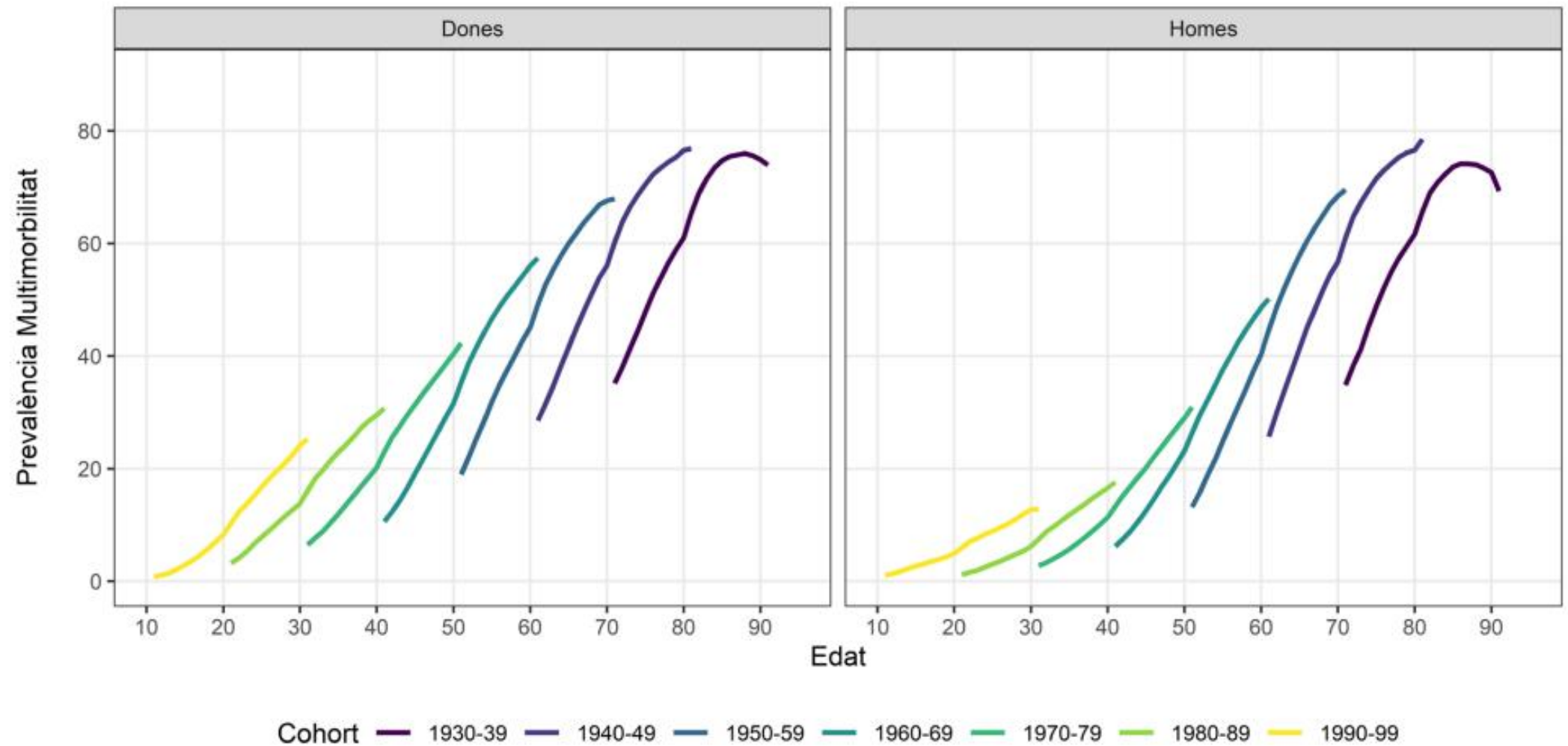
MORBIDITY COMPRESSION PARADIGM

Traditionally, longevity was associated with an inevitable increase in morbidity.

The Morbidity Compression Paradigm (Fries, 1980): as life expectancy increases, the onset of chronic diseases and disabilities can be postponed to a greater extent than the age of death.

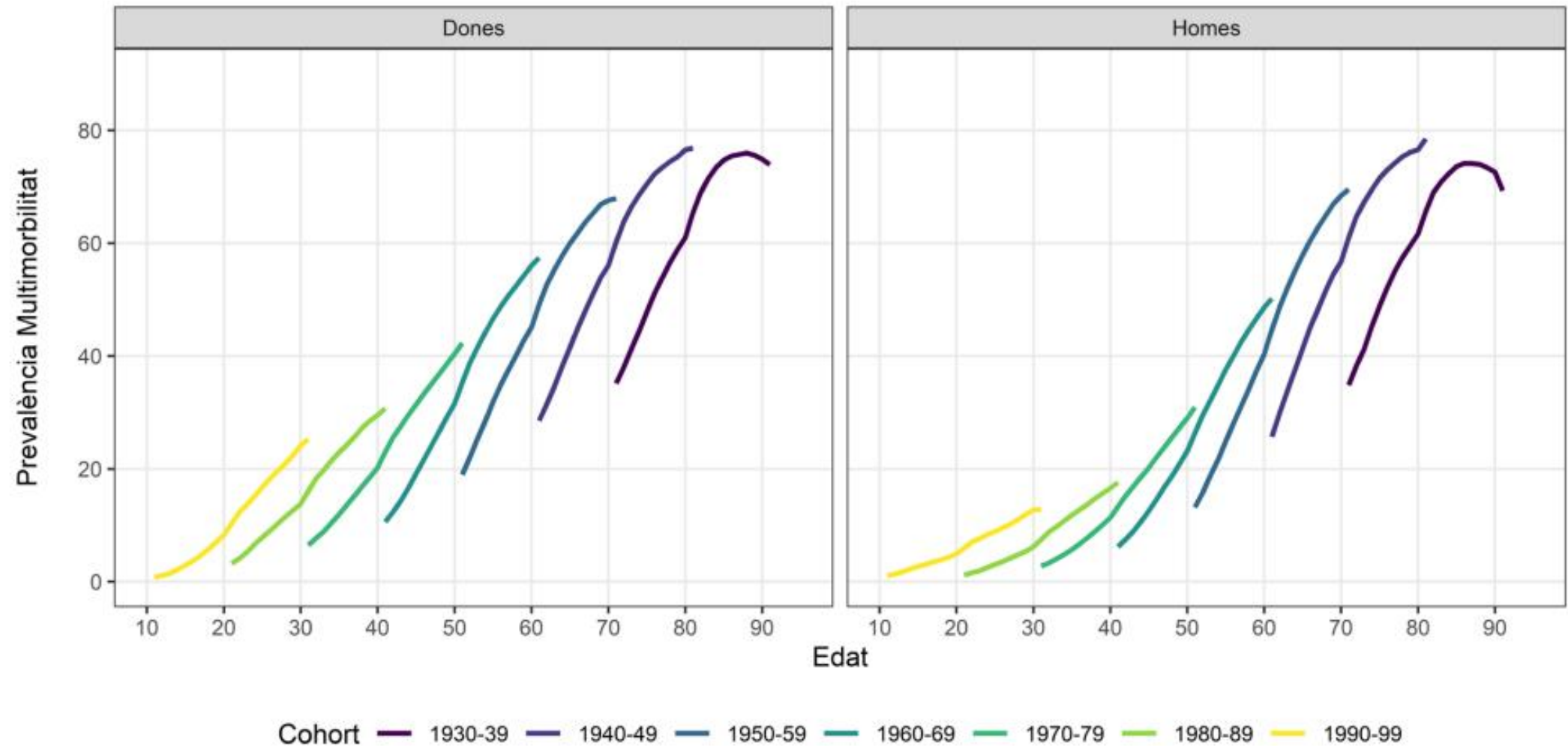
The period of life spent in poor health or with severe disability would be "compressed" into a shorter time frame at the very end of life.

Simply postponing morbidity (delaying when illnesses begin) would improve overall health (Fries, 2012).



Gràfic 1. Tendències en la prevalença de la multimorbiditat complexa per edat i cohort (dones al panell esquerre i homes al dret) entre 2010 i 2021.

Font: Elaboració pròpia a partir de les dades de la cohort HEALIN.



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**Worse health or more
diagnoses?**

**WE ARE LIVING LONGER, BUT ARE WE
LIVING BETTER?**



QUALITY OF LIFE EXPECTANCY

Healthy life expectancy is the average number of years that a person can expect to live in "full health" by considering years lived in less than full health due to disease and/or injury.

There are several ways to estimate the morbidity status. These include:

- Self-reported health status
- Chronic diseases
- Limitations and disabilities
- Basic activities of daily living.

Table 2 Partial life expectancy, healthy life expectancy and proportion of life spent in good self-rated health between the ages of 50 and 75 by socioeconomic position

Partial life expectancy between the ages of 50 and 75							
	Life expectancy	95% CI	Healthy life expectancy	95% CI	Unhealthy life expectancy	95% CI	% spent in good health
Men							
ELSA							
High grade	24.1	24.0, 24.4	19.8	19.4, 20.3	4.3	4.0, 4.7	82.2
Middle grade	23.7	23.4, 24.0	18.2	17.6, 18.9	5.5	4.9, 5.9	76.8
Low grade	22.9	22.6, 23.3	15.5	14.9, 16.1	7.4	7.0, 7.9	67.7
High–low	1.2		4.3		–3.1		14.5

Table 3 Partial life expectancy, chronic disease-free life expectancy and proportion of life spent without chronic disease between the ages of 50 and 75 by socioeconomic position

Partial life expectancy between the ages of 50 and 75							
	Life expectancy ^a	95% CI	Chronic disease-free life expectancy	95% CI	Life expectancy with chronic diseases	95% CI	% spent without chronic disease
Men							
ELSA							
High grade	24.1	23.9, 24.3	13.8	12.7, 14.9	10.3	9.3, 11.3	57.4
Middle grade	23.6	23.3, 24.0	13.2	11.6, 14.4	10.4	9.4, 11.8	56.1
Low grade	23.1	22.8, 23.4	12.8	11.5, 13.8	10.3	9.4, 11.5	55.2
High–low	1.0		1.0		0.0		2.2

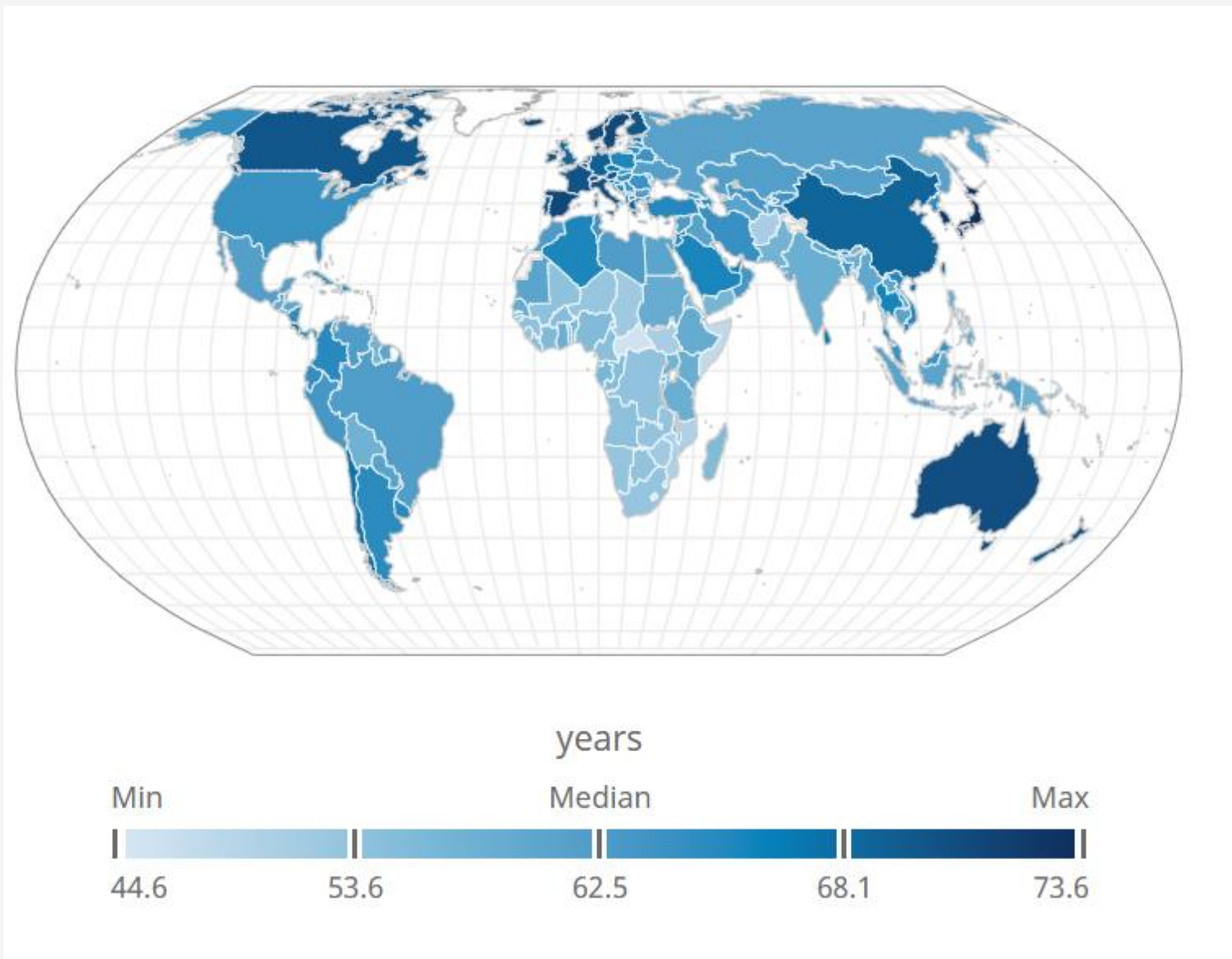
SULLIVAN'S INDEX

The most used method to determine HALE is the Sullivan method, which is computed by subtracting the probable duration of disability or inability to perform important activities from the life expectancy.

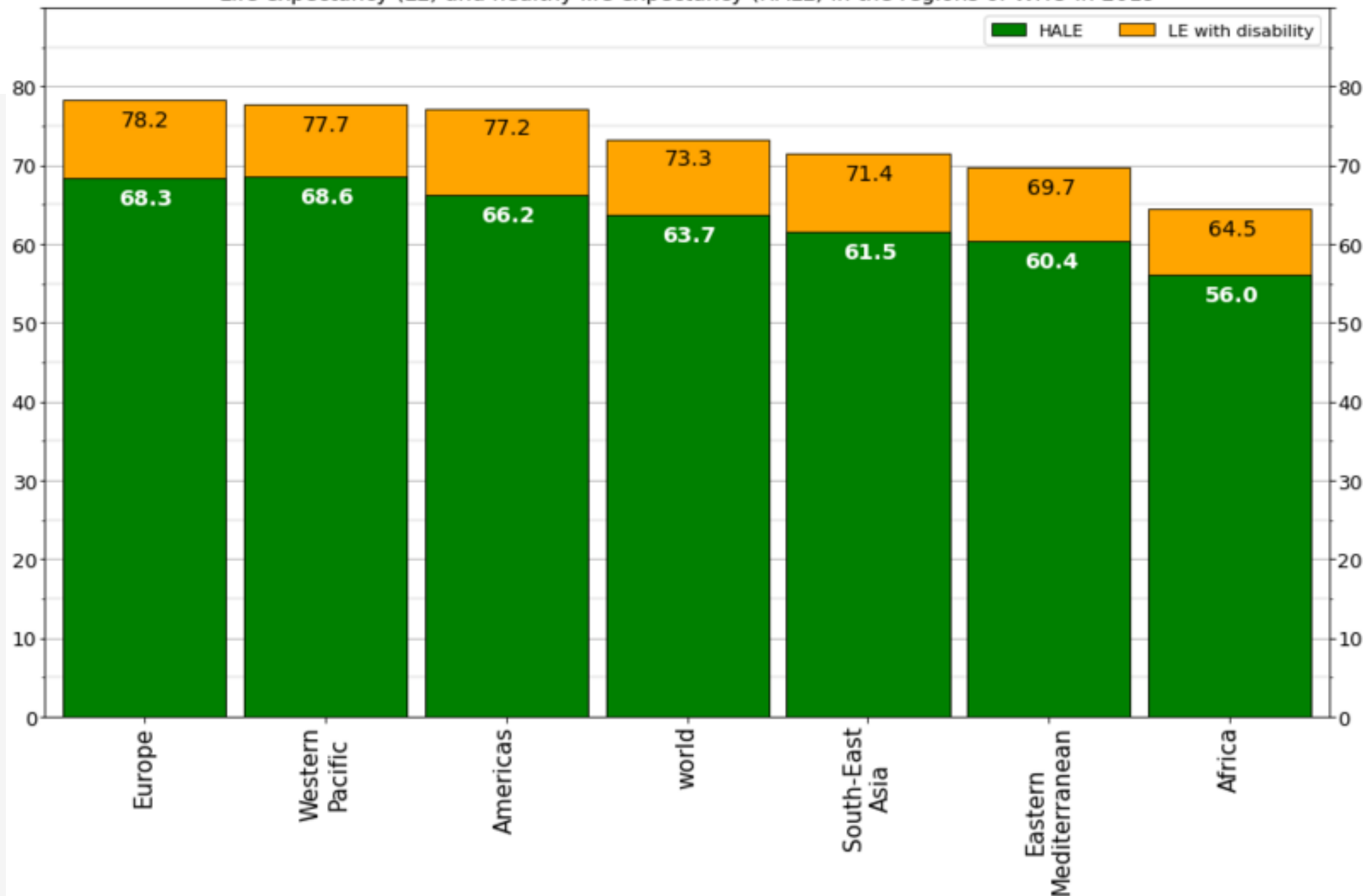
$$HALE_x = \sum \frac{{}_5L_x}{l_x} (1 - {}_5\pi_x)$$

Where ${}_5\pi_x$ is the average level of morbidity between x and $x+5$ years of age.

HEALTHY LIFE EXPECTANCY (WORLD, 2021)

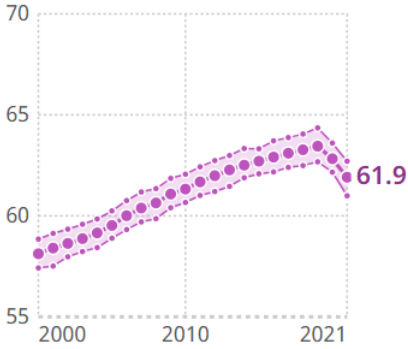


Life expectancy (LE) and healthy life expectancy (HALE) in the regions of WHO in 2019



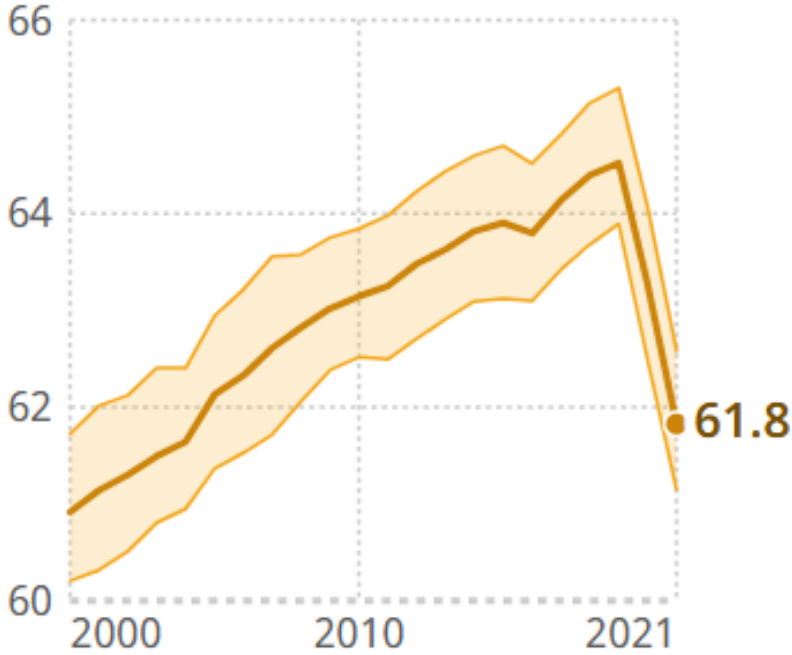
World

2000 – 2021



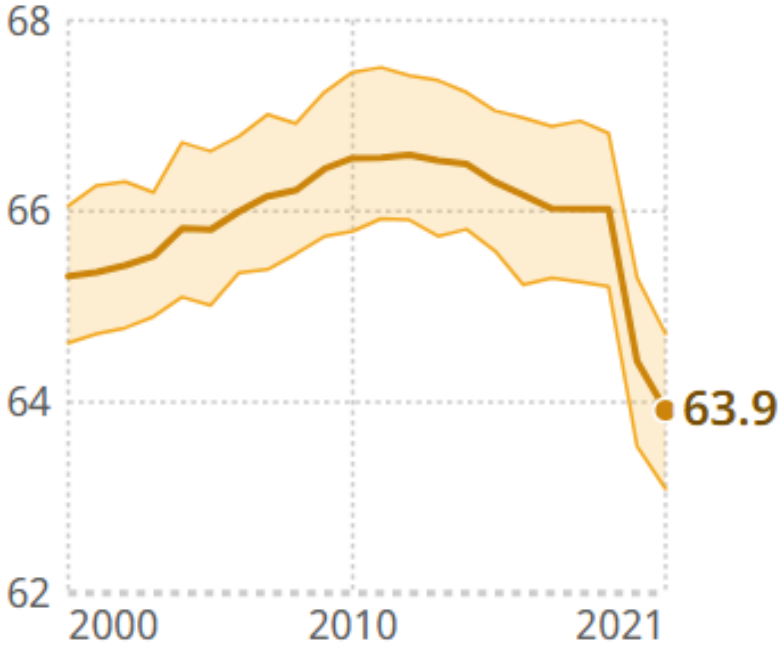
Brazil

Healthy life expectancy (at birth), years **+0.91**

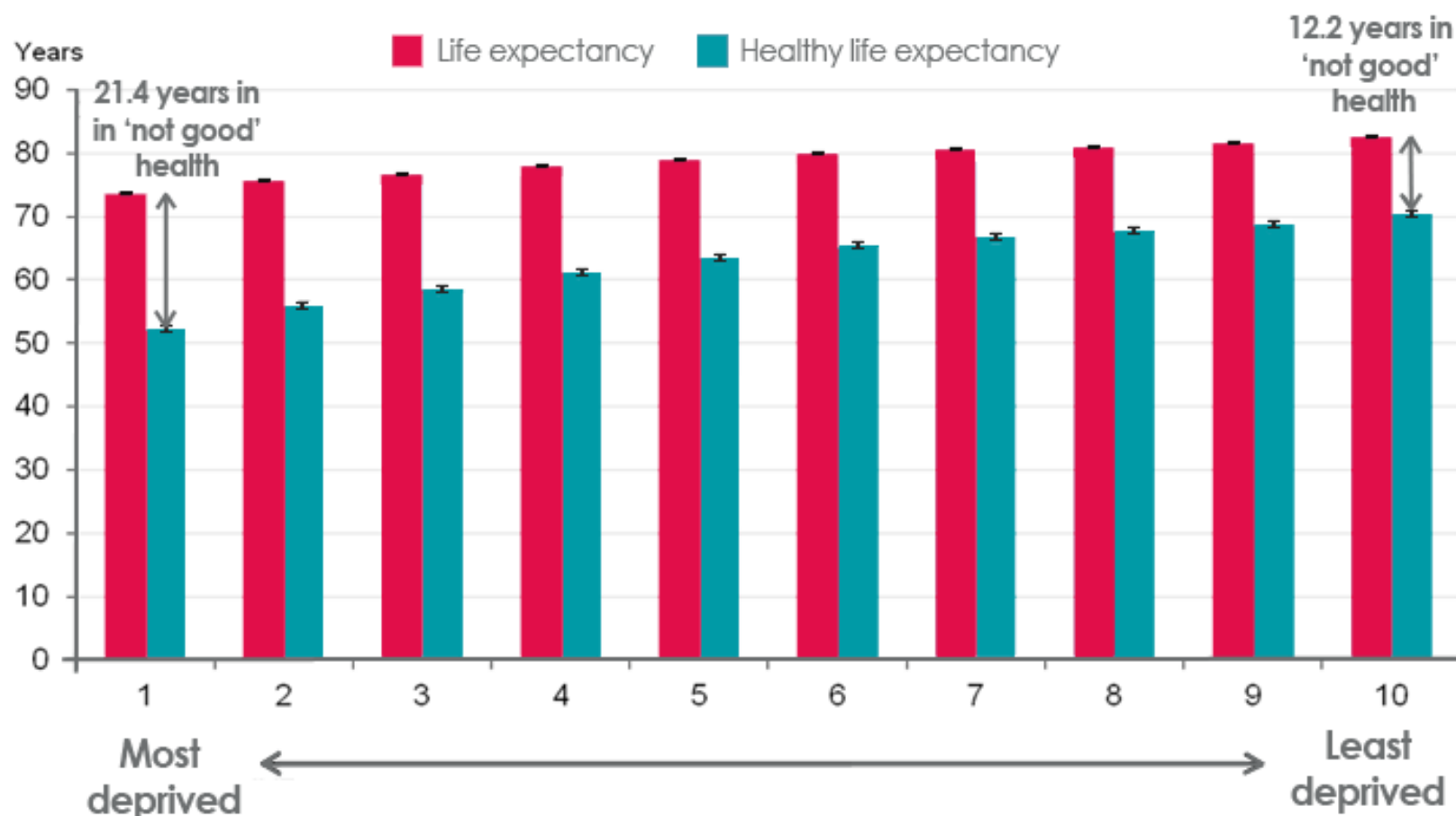


United States of America

Healthy life expectancy (at birth), years **-1.41**

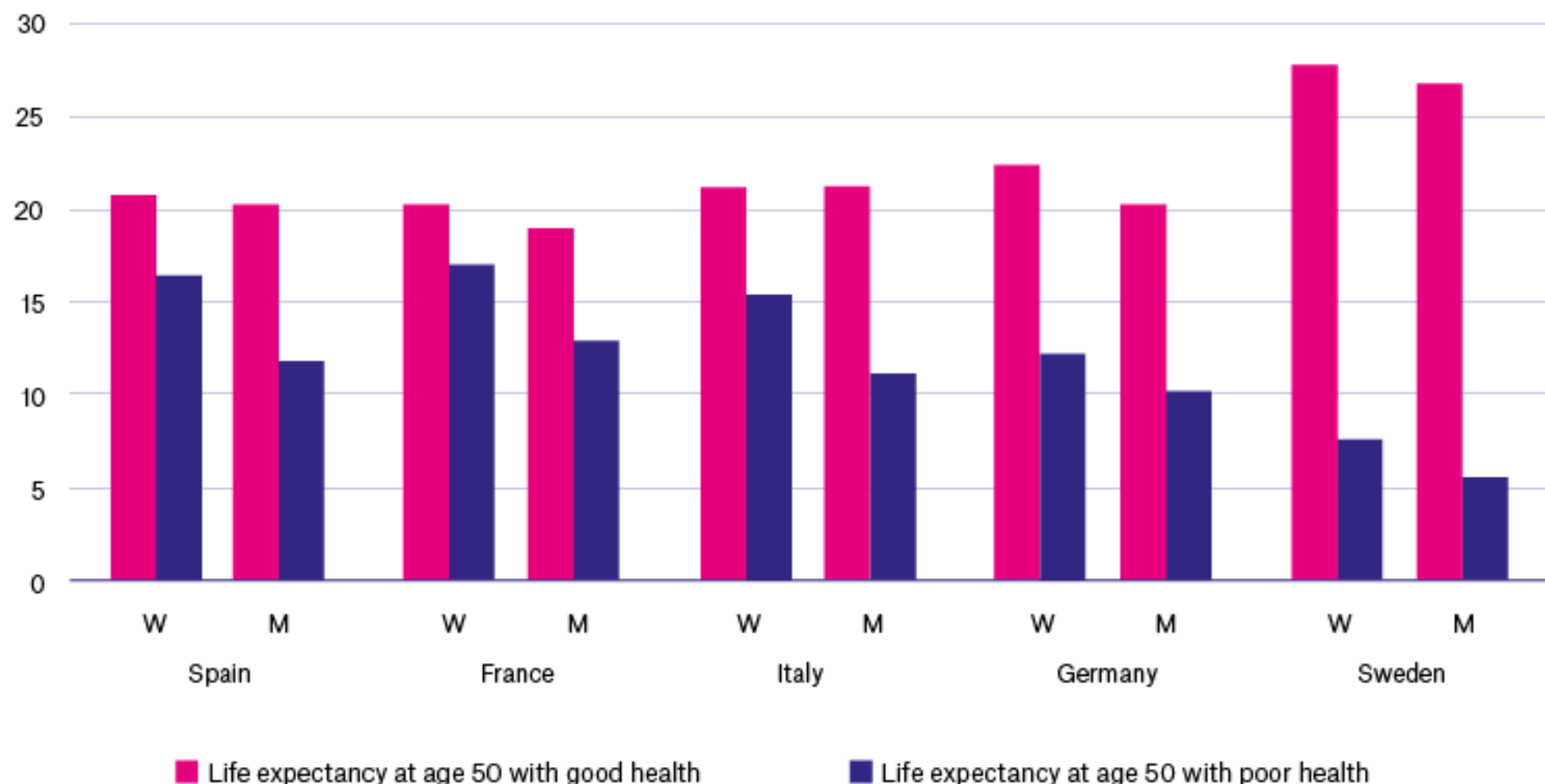


Life expectancy and healthy life expectancy by deciles of deprivation for males in England, 2009-11¹

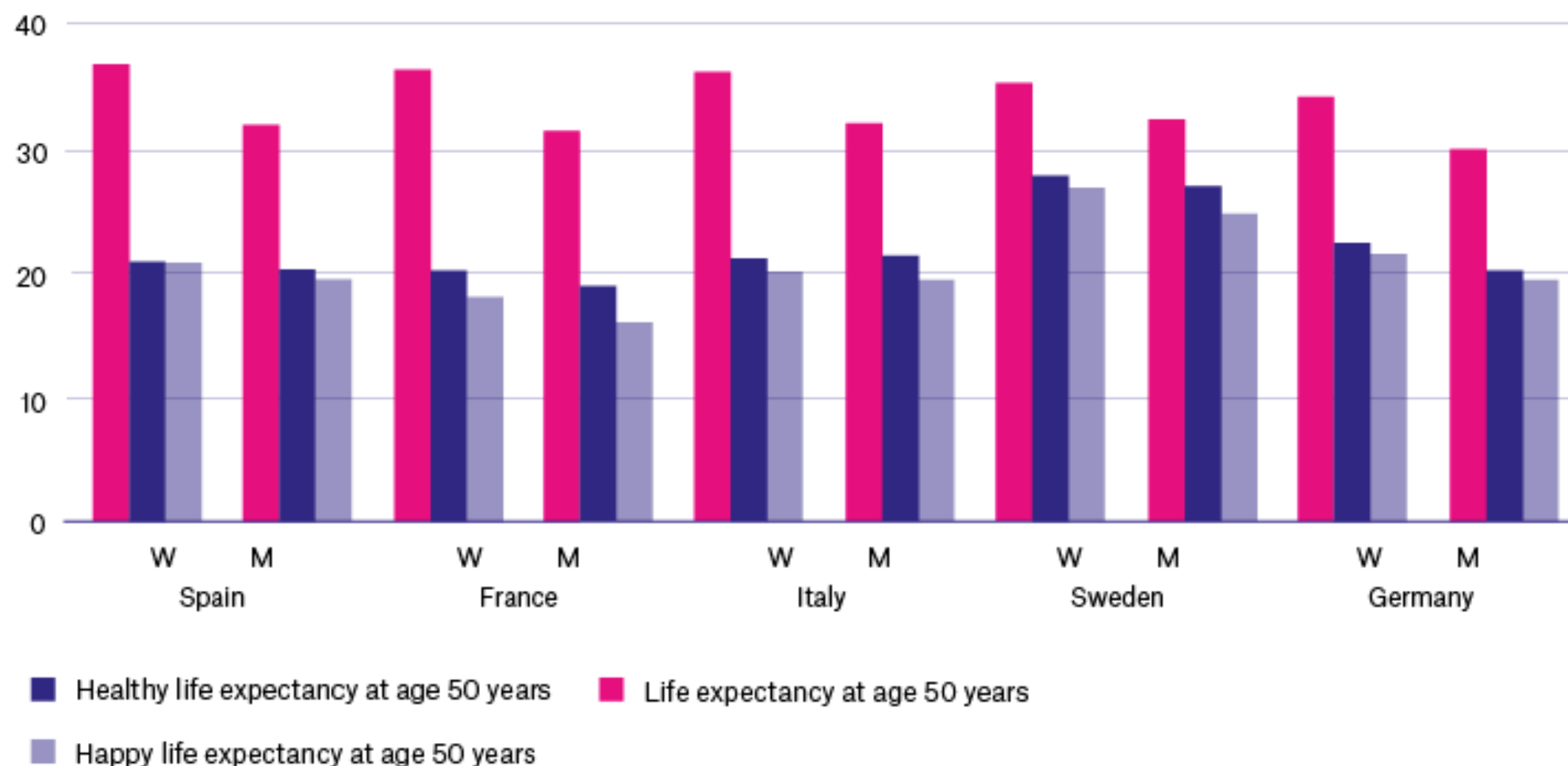


¹ Excludes residents of communal establishments, except NHS housing and students in halls of residence where inclusion takes place at their parents' address.

Graph 1. **Life expectancy with good and poor health at age 50 years by sex, 2016.**



Note: W women; M men.



Note 1: data on life expectancy and healthy life expectancy were obtained from Eurostat. Data on happy life expectancy were obtained from the SHARE database.

Note 2: W: women; M: men.

REDEFINING HEALTHY AGING

Since the first World Report on Aging and Health (2015), the World Health Organization has been reshaping the concept of aging:

healthy aging is more
than just the absence
of disease



developing and
maintaining functional
ability that enables
well-being in older age

HEALTH AND ACTIVE AGING

The term ***Active Aging*** is a comprehensive concept that encompasses the idea of aging well, connecting to other terms such as successful, productive, competent, and healthy aging (Fernández-Ballesteros et al., 2013; Caprara et al., 2013; Fries, 2012).

Active aging embraces multiple domains, such as:

Low probability
of illness and
disability

High physical
fitness

High cognitive
functioning

Positive mood
and ability to
cope with stress

Engagement
with life

ACTIVE AGING INDEX (AAI)

The Active Ageing Index (AAI) measures how well older people are able to live independently, work, or take part in social activities.

It shows the potential older adults have to age actively and healthy.

The index values range from 0 to 100. Higher values mean older men and women are realizing more of their full multifaceted potential.

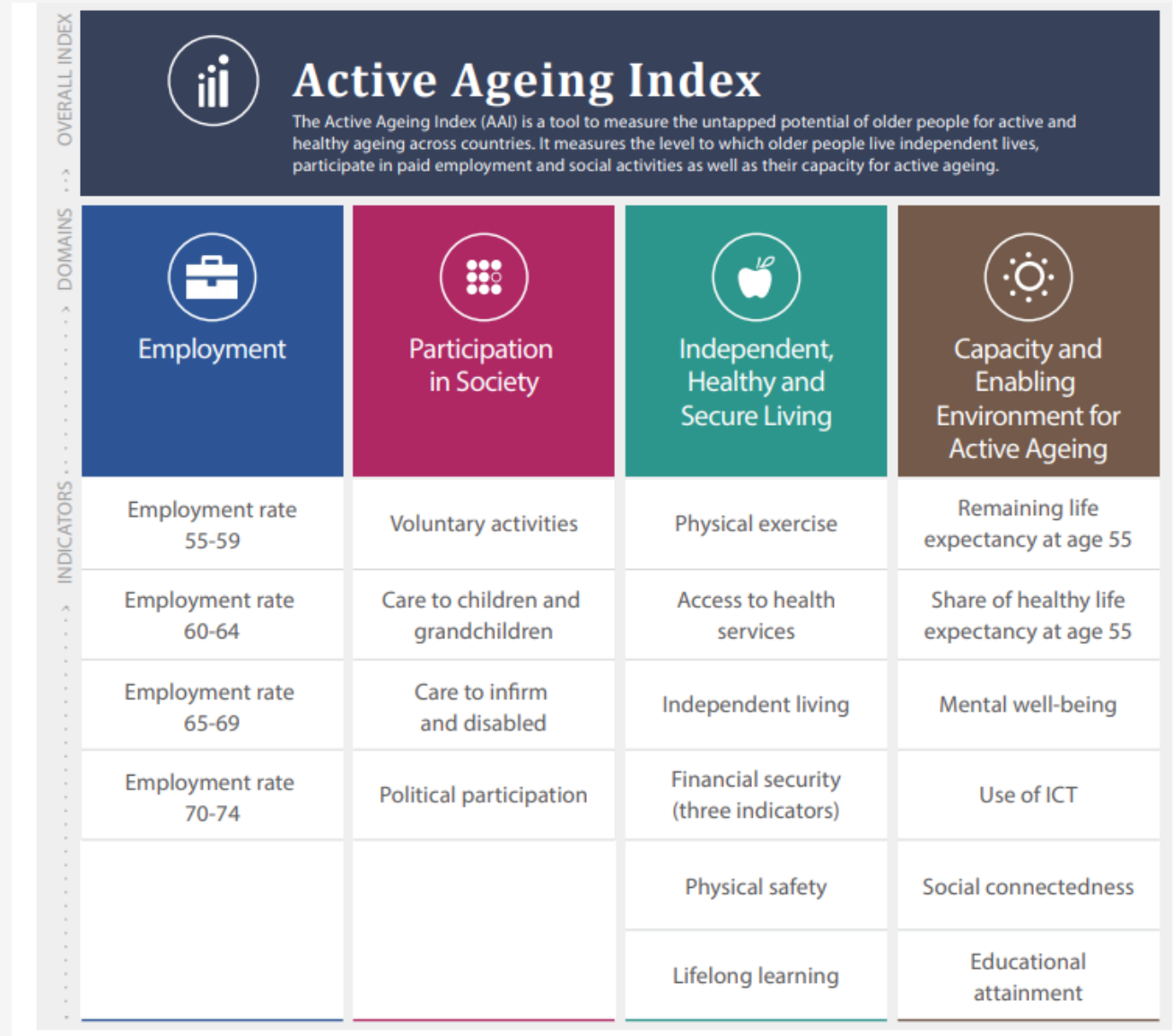
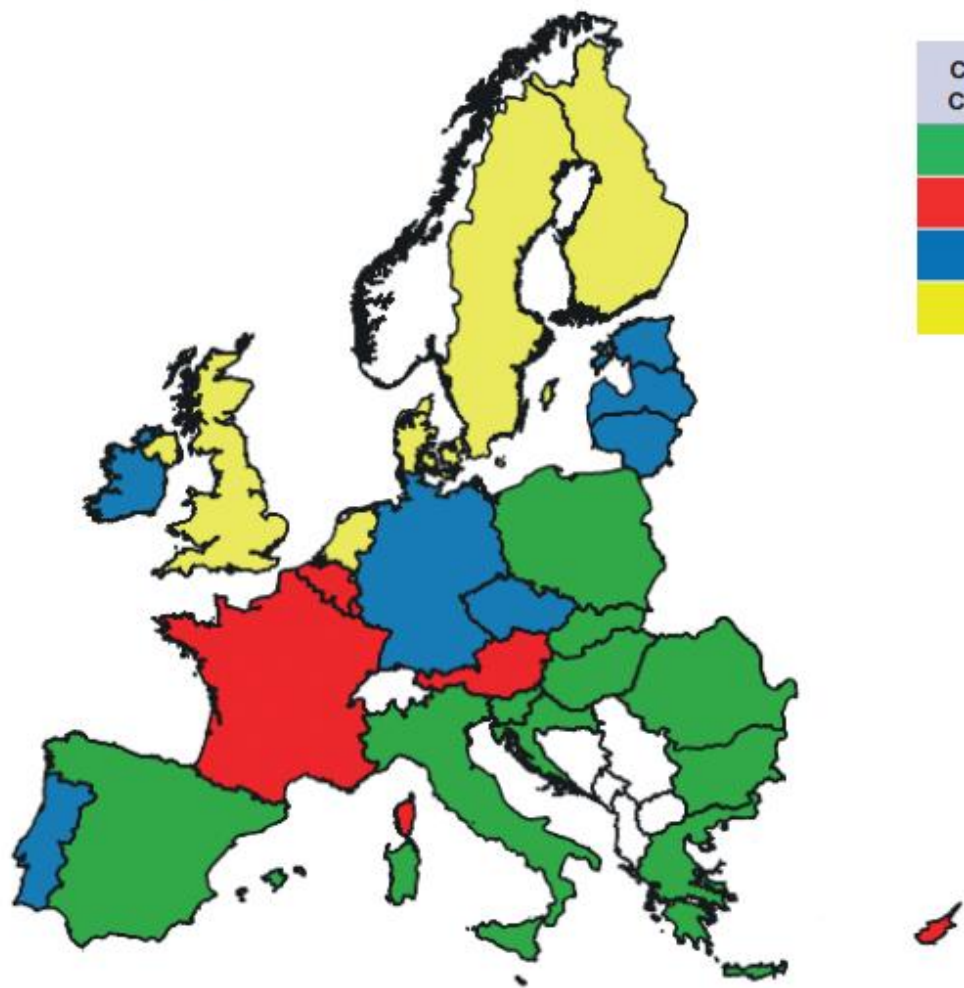


Figure 3: Map of country clusters



Country Clusters	AAI score
1	31.1
2	36.4
3	36.5
4	43.0

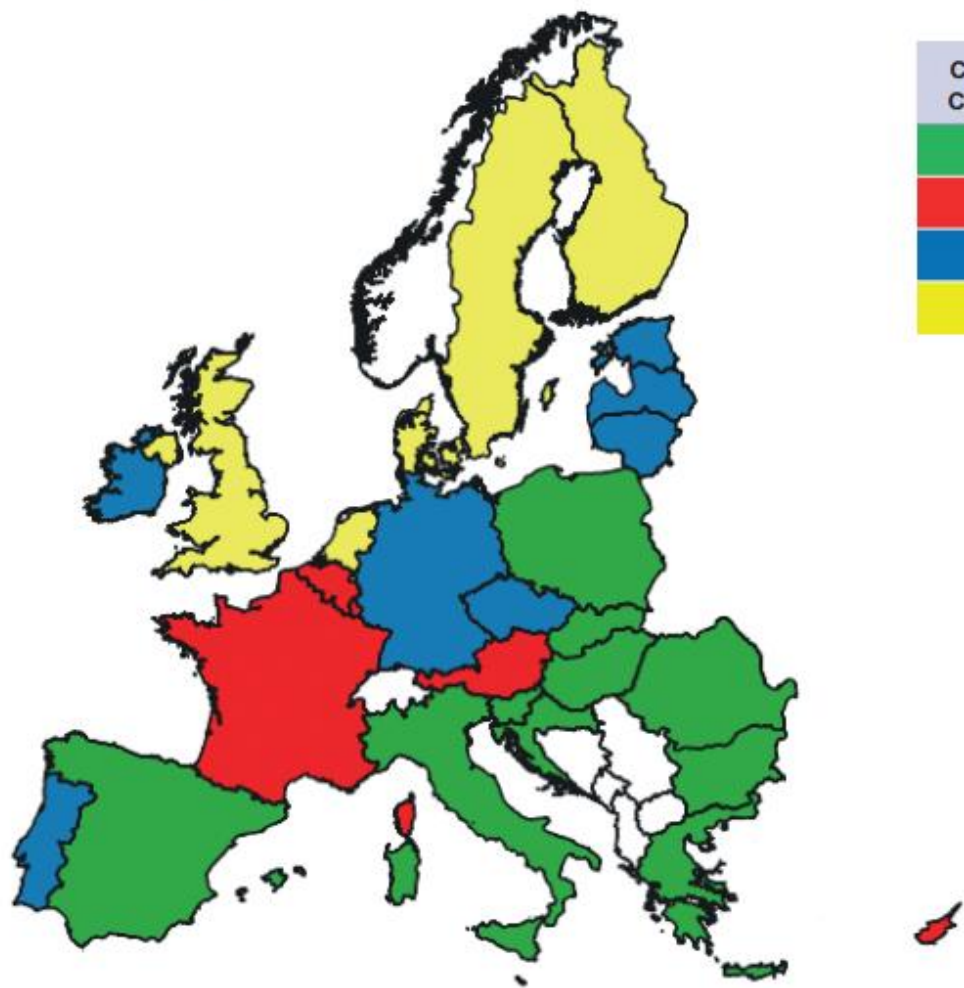
Note: Colours refer to the clusters identified in Table 2.

Table 2: 2018 AAI by country clusters, overall and domain-specific average scores within cluster

Country clusters*	Domain-specific scores				Overall
	Employment	Social participation	Independent, healthy & secure living	Capacity & enabling environment	
1	25.6	14.1	66.9	52.8	31.1
2	25.7	22.7	73.8	60.4	36.4
3	37.5	15.1	68.3	55.9	36.5
4	39.4	23.5	77.6	65.9	43.0
Total	31.1	17.9	70.7	57.5	35.7

*: Clusters contain following countries (colours refer to those shown in Fig. 1):
1. Bulgaria, Croatia, Greece, Hungary, Italy, Poland, Romania, Slovakia, Slovenia, Spain (green);
2. Austria, Belgium, Cyprus, France, Luxembourg, Malta (red);
3. Czech Republic, Estonia, Ireland, Germany, Latvia, Lithuania, Portugal (blue);
4. Denmark, Finland, Netherlands, Sweden, United Kingdom (yellow).
N.B.: for mean standard deviation values see Table A2 in the Annex

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4	39.4	23.5	77.6	65.9	43.0
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N.B.: for mean standard deviation values see Table A2 in the Annex

**HOW CAN WE PROMOTE
THE HEALTH AND ACTIVE AGING?**

PROMOTING HEALTHY AND ACTIVE AGING

Employment
• Incentivize hiring and retention of older workers (e.g. tax benefits). • Promote flexible work arrangements and age-friendly workplaces. • Invest in lifelong learning and digital skills training. • Support later-life career transitions and entrepreneurship. • Enforce anti-age discrimination laws.

Social Participation
• Support intergenerational and senior volunteer programs. • Facilitate civic engagement in local councils and community groups. • Expand access to cultural, artistic, and recreational activities. • Promote digital inclusion to reduce social isolation. • Strengthen civil society organizations led by and for older people.

Independent Living
• Subsidize home adaptations for safety and accessibility. • Develop age-friendly housing and co-living options. • Ensure accessible and affordable public transportation. • Expand in-home and community-based care services. • Support informal caregivers with training and respite options.

**WE ARE LIVING LONGER, BUT ARE WE
LIVING BETTER?**

WHAT ARE THE IMPLICATIONS OF AGING FOR HEALTHCARE SYSTEMS?

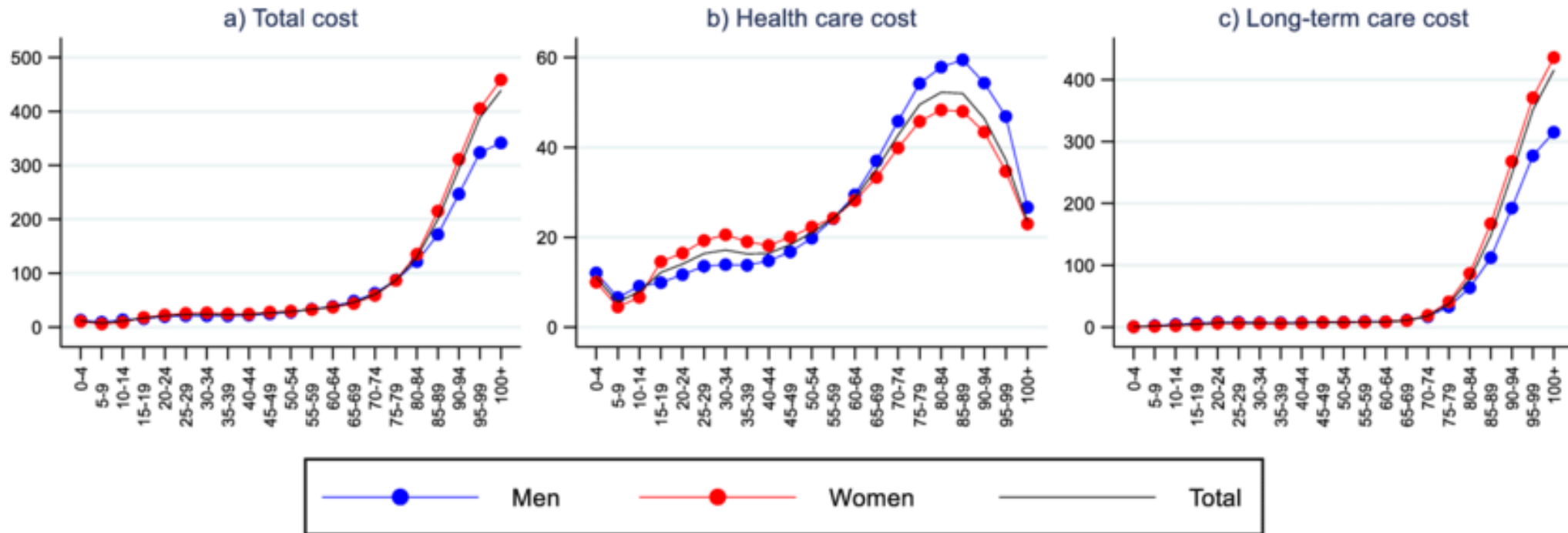
FUTURE CHALLENGES FOR HEALTH SYSTEMS

- Increase in healthcare spending
- Incorporation of new medical technologies
- Shortage of qualified labor (nurses, doctors etc)
- Climate changes
- Unhealthy lifestyle habits
- Mental health and its effects on chronic illnesses
- Socioeconomic inequalities

FUTURE CHALLENGES FOR HEALTH SYSTEMS

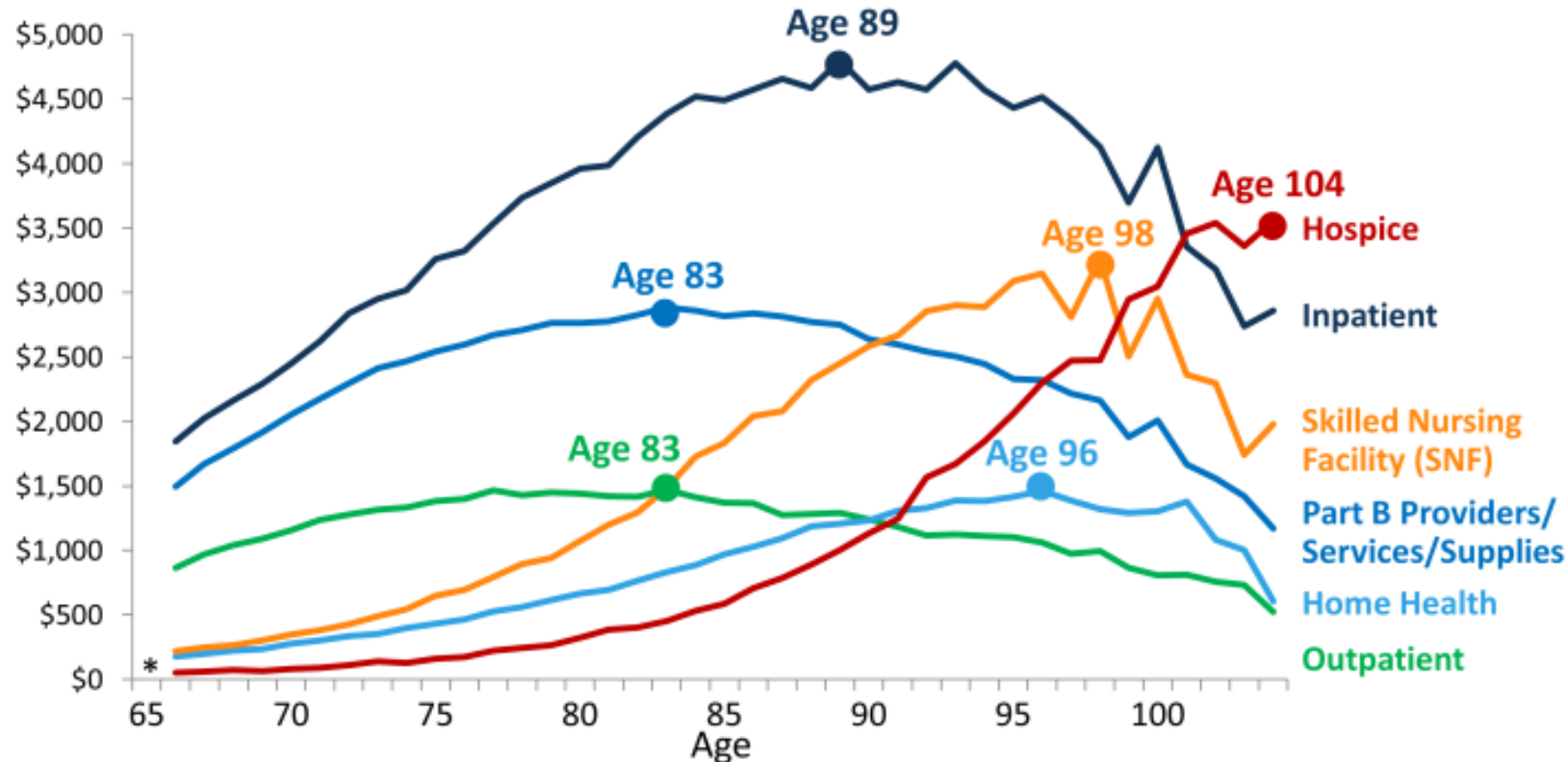
- **Increase in healthcare spending**
- Incorporation of new medical technologies
- **Shortage of qualified labor (nurses, doctors etc)**
- Climate changes
- Unhealthy lifestyle habits
- Mental health and its effects on chronic illnesses
- **Socioeconomic inequalities**

Per capita costs by age and gender. Total cost, healthcare cost, and long-term care cost – Norway (2010)



Kalseth, J., & Halvorsen, T. (2020). Health and care service utilisation and cost over the life-span: a descriptive analysis of population data. *BMC health services research*, 20, 1-14.

Peaks in Medicare per capita spending by type of service for traditional Medicare beneficiaries over age 65, 2011

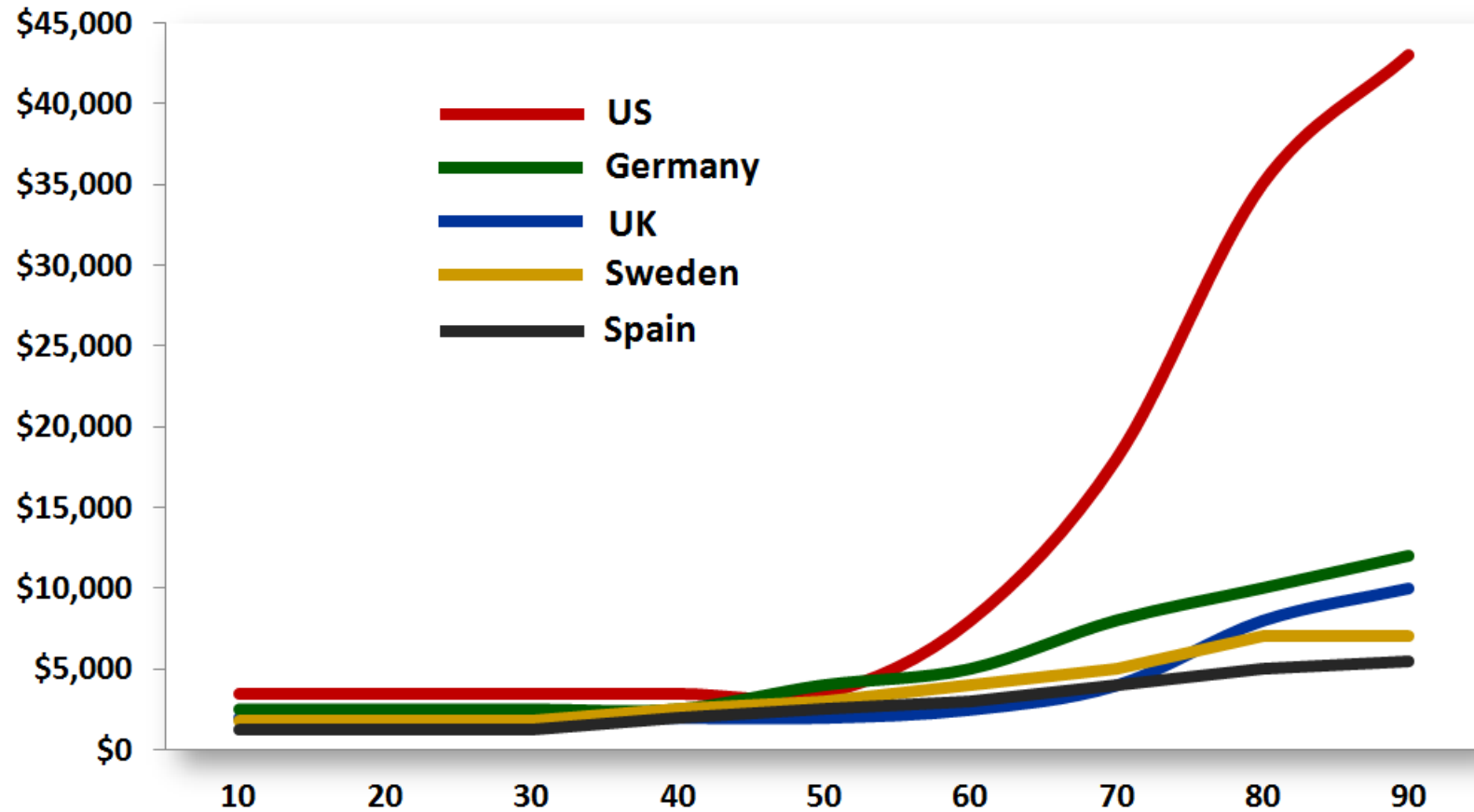


NOTE: Analysis excludes beneficiaries with Medicare Advantage. *Analysis excludes people age 65 because some of these beneficiaries are enrolled for less than a full year; therefore, a full year of Medicare spending data is not available for all people at this year of age.
SOURCE: Kaiser Family Foundation analysis of a 5 percent sample of Medicare claims from the Chronic Conditions Data Warehouse, 2011.

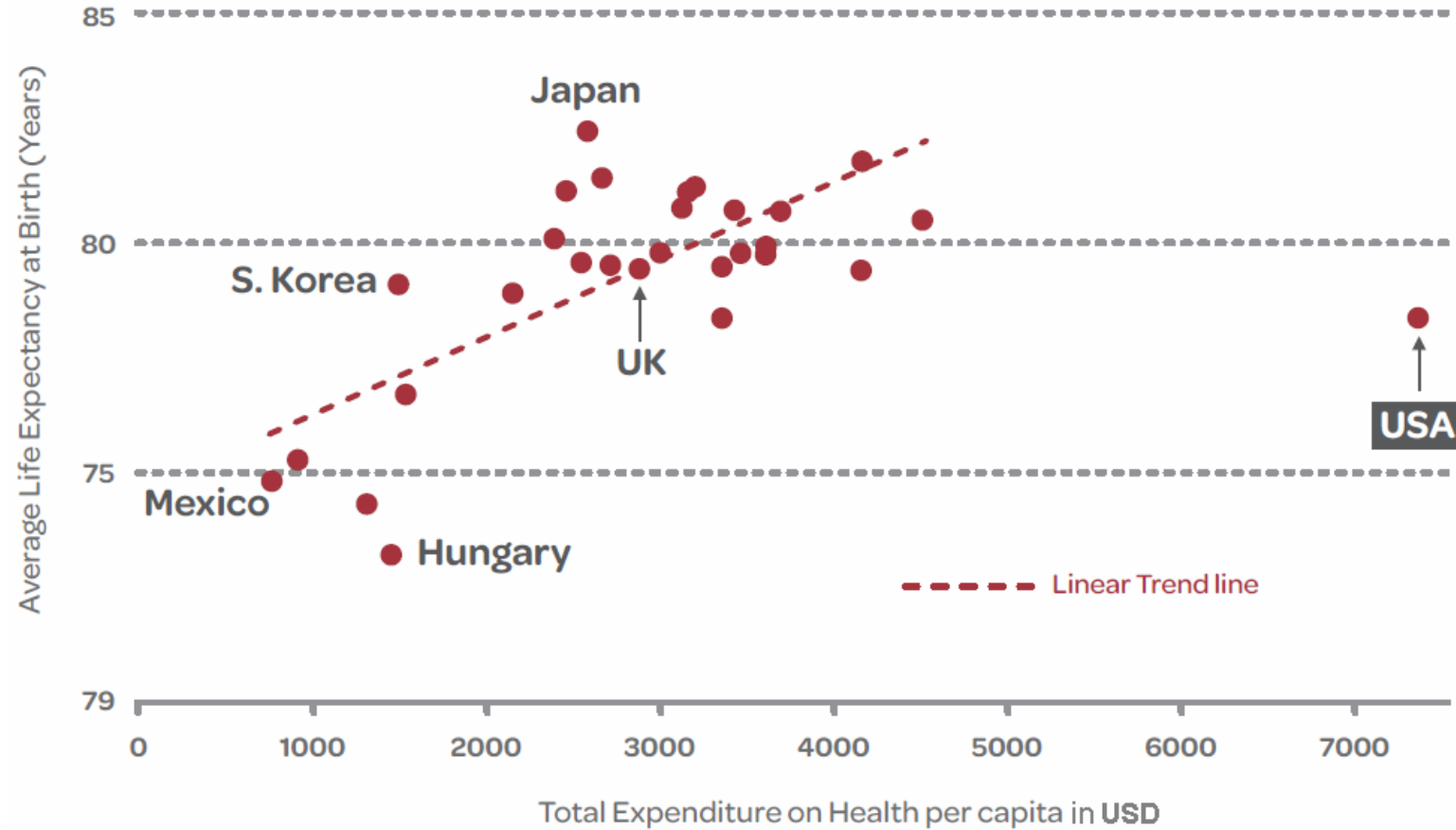


Neuman, T., Huang, J., Probst, K., et al. (2015). *The rising cost of living longer: Analysis of Medicare spending by age for beneficiaries in traditional Medicare*. Kaiser Family Foundation. <https://www.kff.org/medicare/report/the-rising-cost-of-living-longer-analysis-of-medicare-spending-by-age-for-beneficiaries-in-traditional-medicare/>

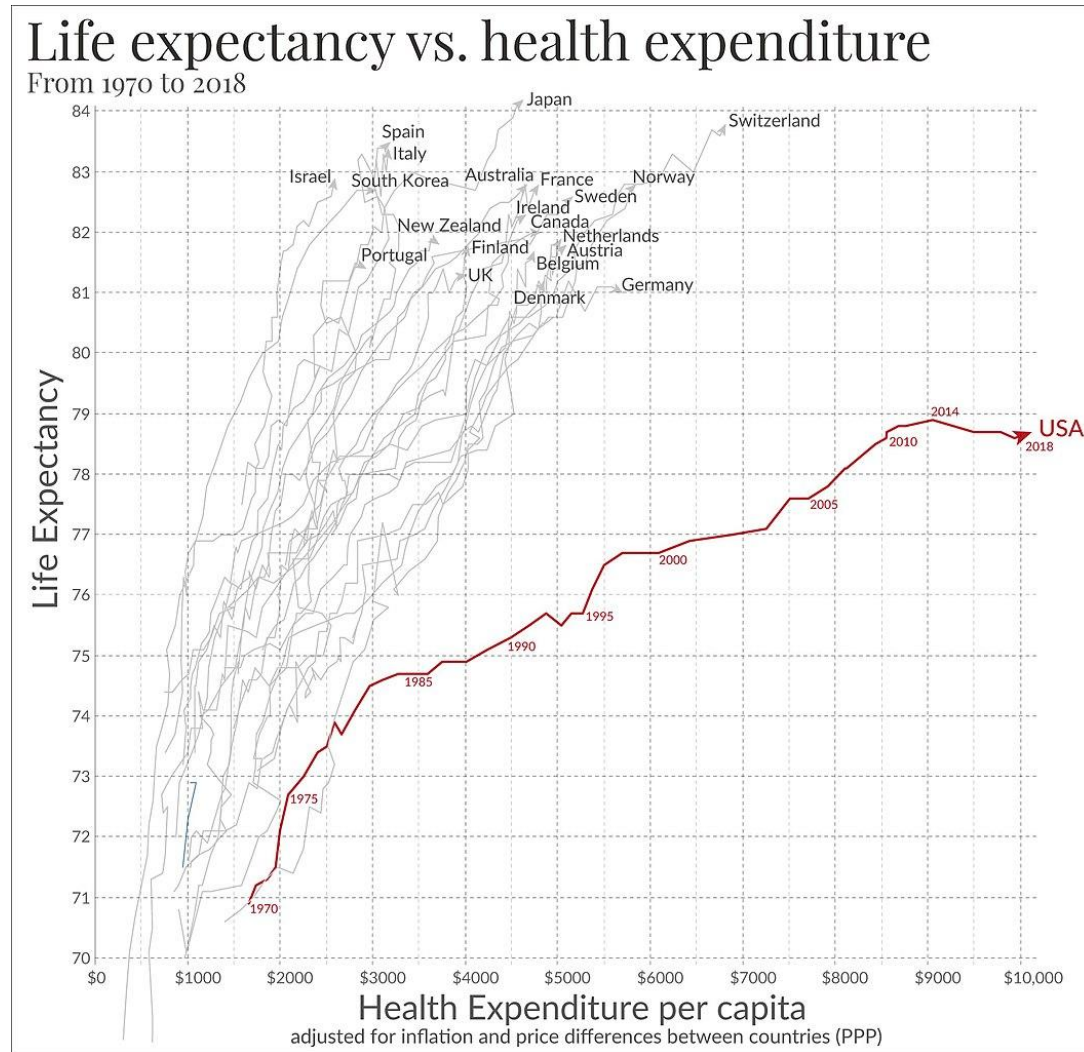
Annual Per Capita Healthcare Costs by Age



Healthcare Spending per capita vs. Average Life Expectancy Among OECD Countries



https://www.forbes.com/sites/danmunro/2012/12/30/2012-the-year-in-healthcare-charts/?goback=%2Egde_700187_member_199832527

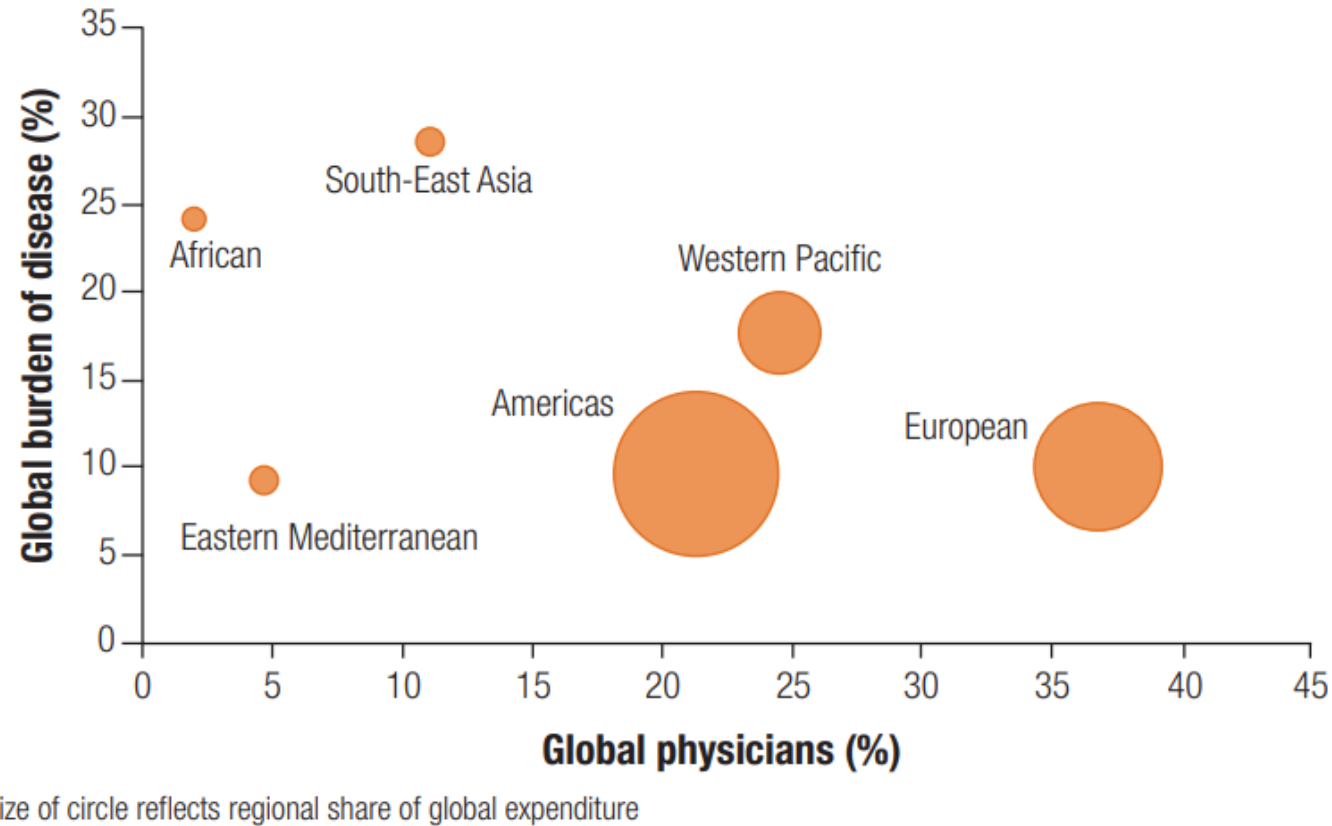


Roser, M. (2017). Link between health spending and life expectancy: US is an outlier. *Our World in Data*, 1.

FUTURE CHALLENGES FOR HEALTH SYSTEMS

- Increase in healthcare spending
- Incorporation of new medical technologies
- **Shortage of qualified labor (nurses, doctors etc)**
- Climate changes
- Unhealthy lifestyle habits
- Mental health and its effects on chronic illnesses
- Socioeconomic inequalities

Fig. 2. **Physician distribution by burden of disease and health expenditure, by WHO region, 2004**



Source: *World health statistics 2006*.¹³

Scheffler, R. M., Liu, J. X., Kinfu, Y., & Dal Poz, M. R. (2008). Forecasting the global shortage of physicians: an economic-and needs-based approach. *Bulletin of the World Health Organization*, 86(7), 516-523B.

Table 4. Nurse demand, supply and surplus/shortage projections by country, 2030

Country	Demand (95% confidence interval)	Supply	Surplus/ shortage	Surplus/shortage (as % of supply)
Australia	427,567 (396,088, 461,546)	299,067	-128,499	-43.0
Austria	101,760 (92,605, 111,819)	78,790	-22,969	-29.2
Canada	564,304 (496,227, 641,719)	446,703	-117,600	-26.3
Czech Republic	123,864 (112,847, 135,956)	86,625	-37,239	-43.0
Denmark	126,084 (116,575, 136,369)	136,951	10,866	7.9
France	838,253 (772,869, 909,169)	873,940	35,687	4.1
Germany	1,426,505 (1,275,953, 1,594,822)	1,452,273	25,768	1.8
Hungary	73,663 (68,995, 78,647)	68,625	-5,039	-7.3
Iceland	7800 (7,027, 8,657)	6,793	-1,007	-14.8
Israel	67,856 (63,704, 72,279)	46,619	-21,237	-45.6
Korea	413,528 (347,232, 492,481)	385,908	-27,620	-7.2
Luxembourg	9211 (8,862, 9,575)	11,867	2,656	22.4
Mexico	594,561 (544,471, 649,260)	495,758	-98,804	-19.9
Netherlands	365,078 (325,031, 410,060)	260,151	-104,928	-40.3
Poland	383,278 (345,220, 425,530)	237,304	-145,973	-61.5
Slovakia	57,126 (51,125, 63,832)	30,218	-26,909	-89.0
Slovenia	26,355 (23,411, 29,669)	20,977	-5,378	-25.6
Spain	267,875 (236,594, 303,292)	318,712	50,837	16.0
Sweden	151,110 (141,650, 161,203)	140,702	-10,408	-7.4
Switzerland	245,736 (230,076, 262,461)	212,657	-33,079	-15.6
Turkey	221,414 (190,529, 257,305)	305,367	83,953	27.5
United Kingdom	824,163 (775,748, 875,599)	727,291	-96,872	-13.3
United States	6,093,332 (5,557,991, 6,680,236)	4,286,071	-1,807,261	-42.2
Total (23 countries)	13,410,421 (12,180,830, 14,771,486)	10,929,369	-2,481,052	-22.7

Scheffler, R. M., & Arnold, D. R. (2018). Projecting shortages and surpluses of doctors and nurses in the OECD: what looms ahead. *Health Economics, Policy and Law*, 1–17.

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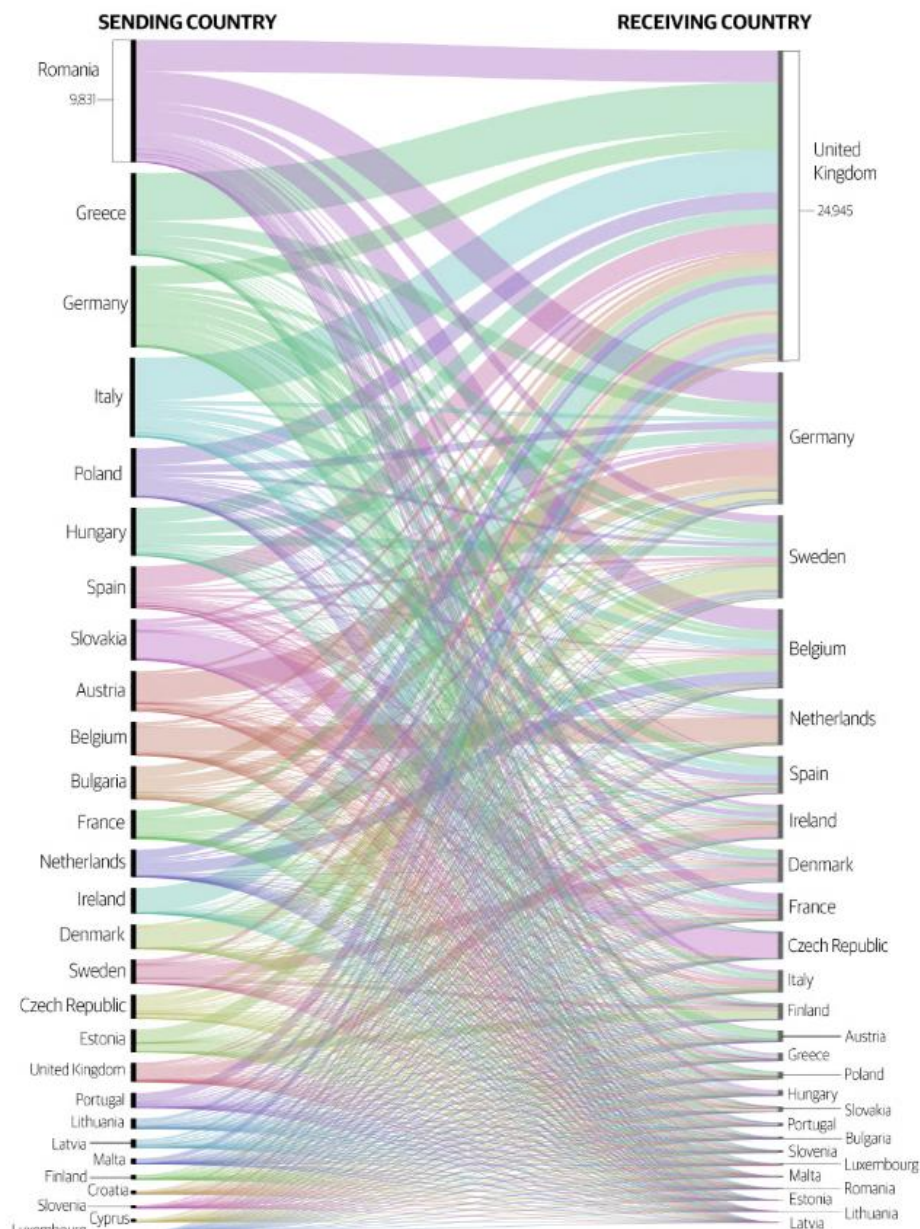
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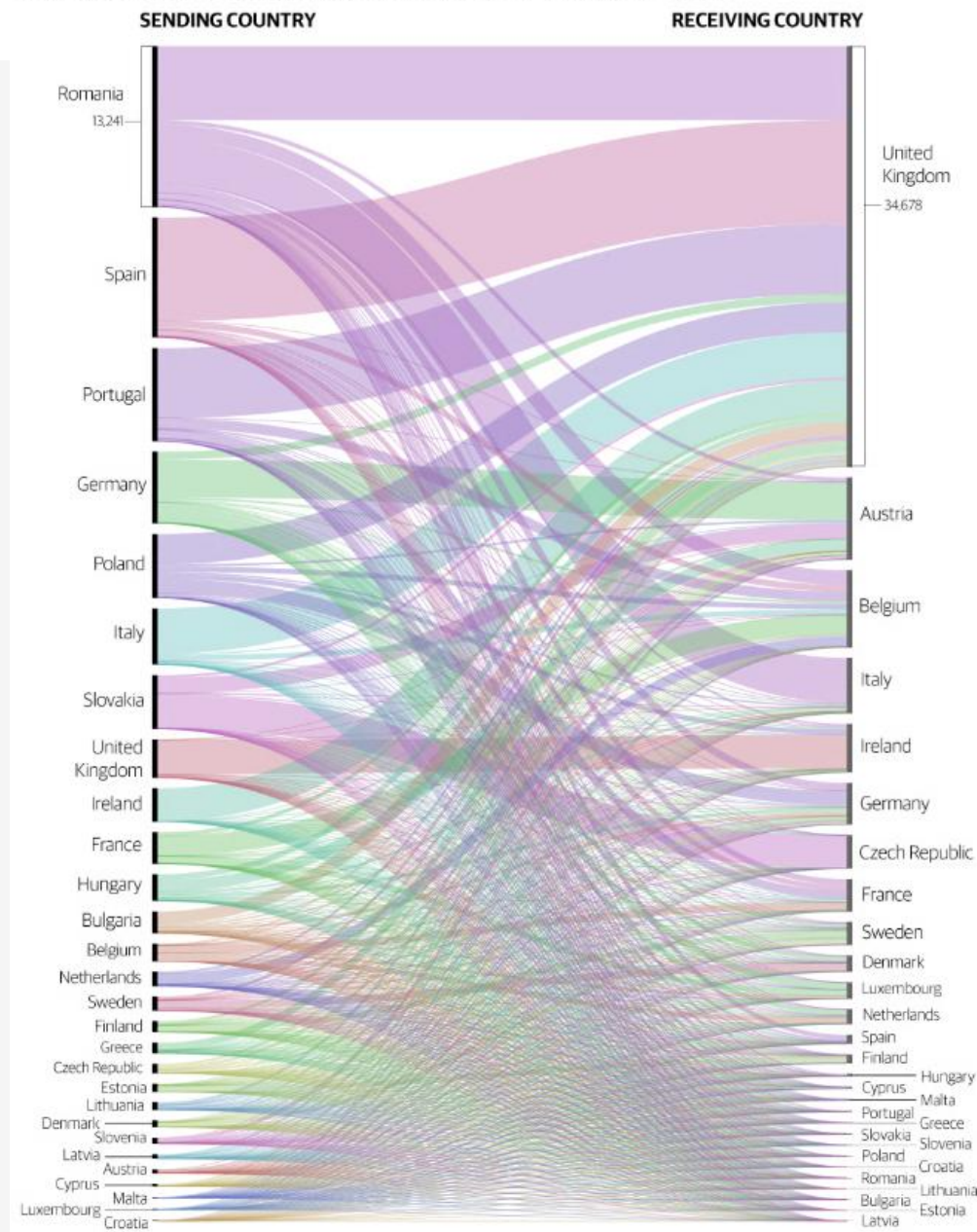
WHERE DO THEY COME FROM, WHERE DO THEY GO?

72,314 doctors from EU countries applied to work elsewhere in the bloc between 1997 and 2016.*
Here are the countries they sought to leave and those they wanted to join.



CARE ON THE MOVE

73,067 nurses from EU countries applied to work elsewhere in the bloc between 1997 and 2016.*
Here are the countries they sought to leave and those they wanted to join.



FUTURE CHALLENGES FOR HEALTH SYSTEMS

- Increase in healthcare spending
- Incorporation of new medical technologies
- Shortage of qualified labor (nurses, doctors etc)
- Climate changes
- Unhealthy lifestyle habits
- Mental health and its effects on chronic illnesses
- **Socioeconomic inequalities**

INEQUALITY, HEALTH AND MORTALITY

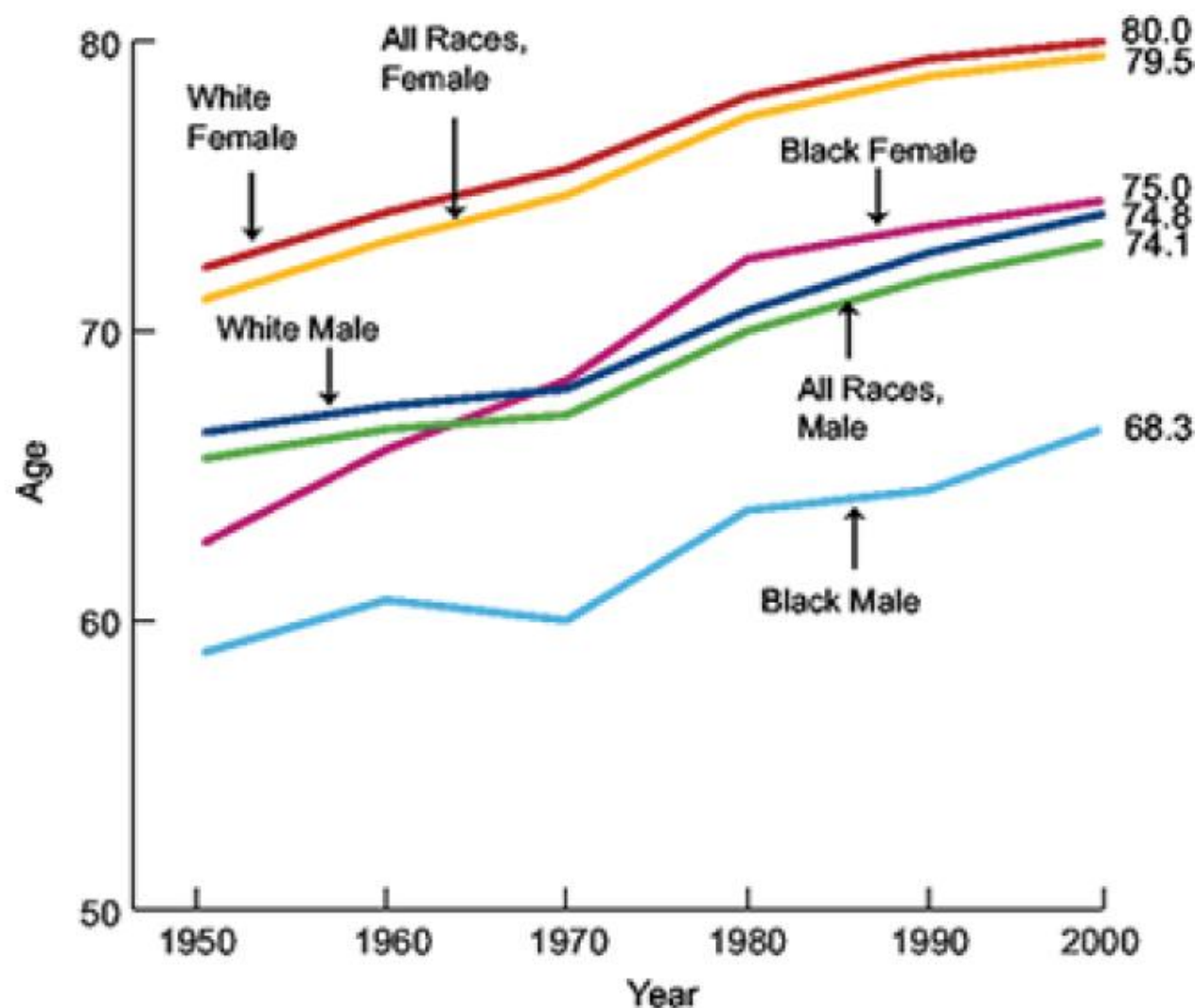
In most countries, people with higher educational levels, higher incomes, and better occupations tend to have better health and lower mortality.

Factors related to socioeconomic status include:

- Poorer living and nutritional conditions
- Access to health services
- Health conditions during pregnancy and childhood (Case, Barker, among others)
- Access to information on healthy lifestyle habits
- Stress accumulation
- Macroeconomic effects of inequality
- Longevity and long-term planning

Life Expectancy At Birth, by Sex and Race, 1950-2000

Source (II.1, II.2): National Vital Statistics System

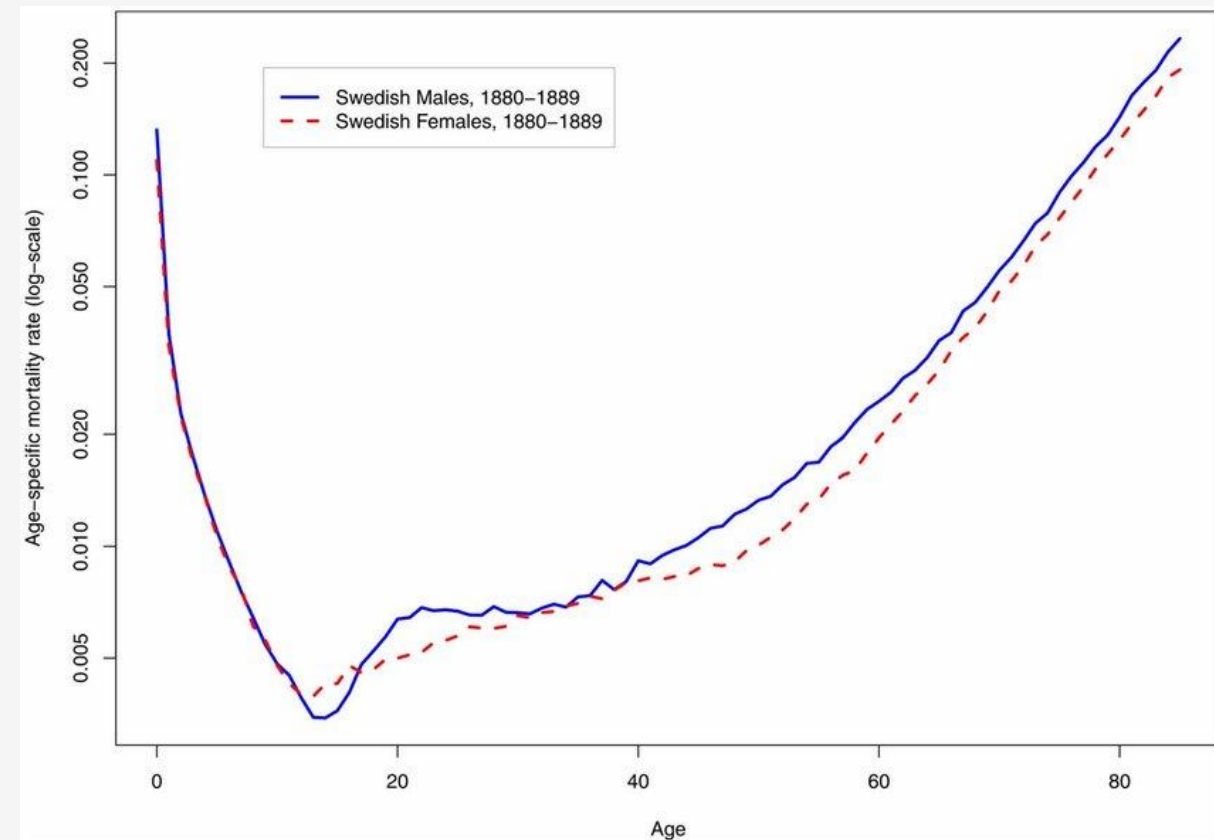


Note: Life expectancy calculations are not available for other racial or ethnic groups.

GENDER GAPS IN HEALTH

In most countries in the world, women have a significantly greater hope of life than men.

In addition to biological differences between the sexes, other socioeconomic and psychological factors can explain differences in mortality between men and women, such as violence and the adoption of less healthy lifestyles by men (Case & Paxson, 2005).

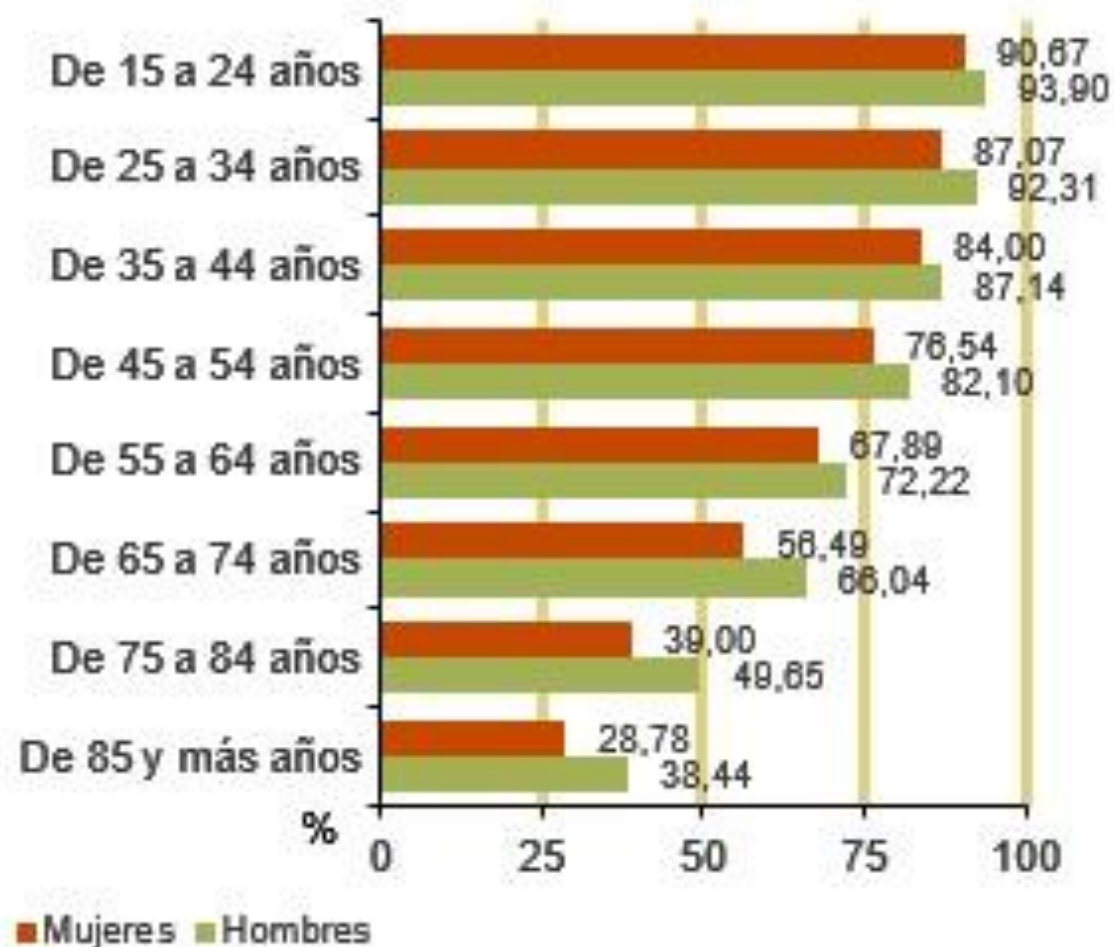


GENDER GAPS IN HEALTH

On the other hand, women suffer from a higher prevalence of chronic illnesses, such as arthritis, and mental health problems, such as depression, although these illnesses cannot be as lethal as those that most commonly affect men.

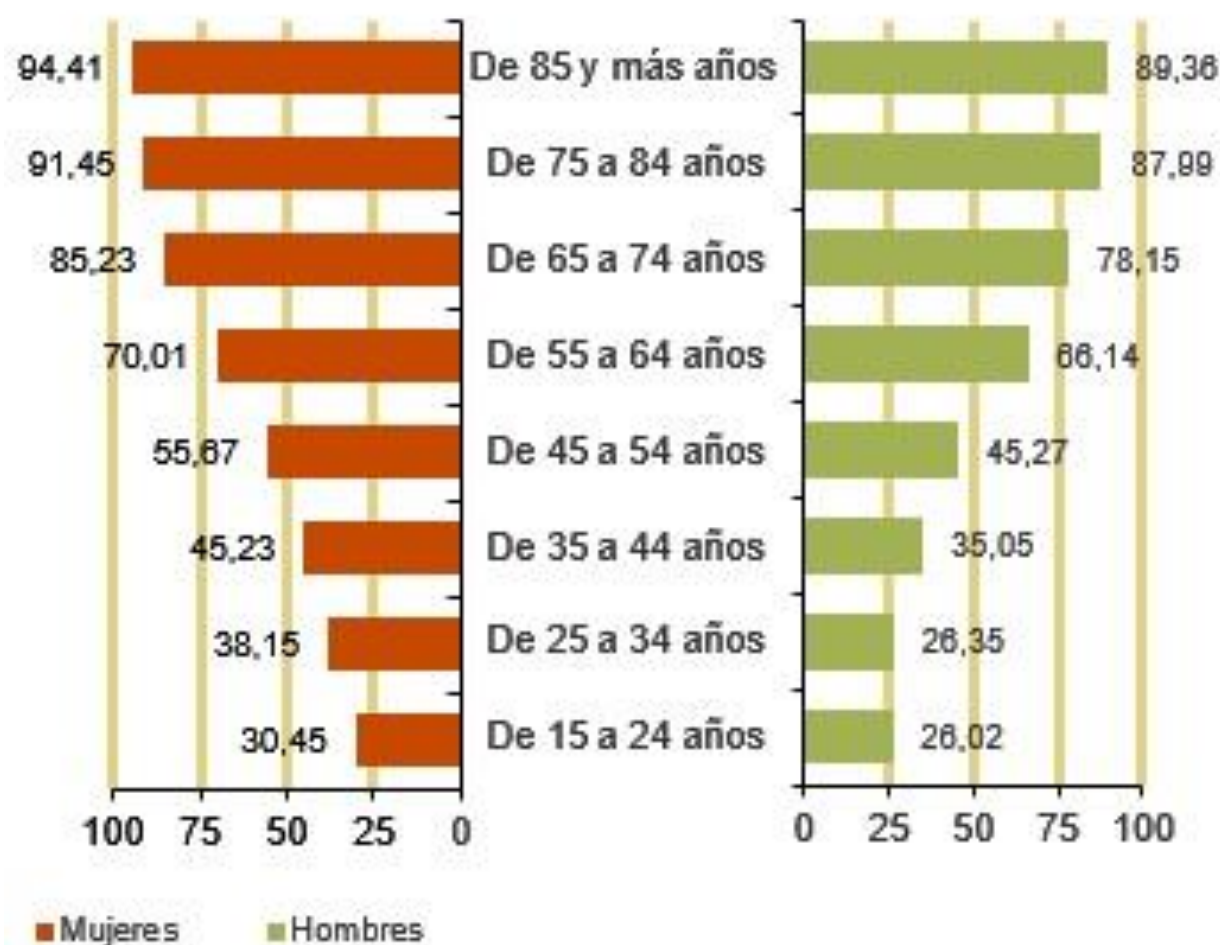


Estado de salud percibido: muy bueno o bueno. 2020
(% población de 15 y más años)



Fuente: Encuesta Europea de Salud en España 2020. INE-MSCBS

% Población de 15 y más años con alguna enfermedad o problema de salud crónico percibido. 2020



Fuente: Encuesta Europea de Salud en España 2020. INE-MSCBS